SUPPLEMENTARY DIGITAL MATERIAL 1

Supplementary Table I.—Overview of the included studies.

YEAR	1ST AUTHOR	COUNTRY	PARTICIPANTS			RANDOMIZED PATIENTS (IG: n; CG: n)			OUTCOME MEASURES	INTERVENTION DESCRIPTION (method section)	COMPARISON DESCRIPTION (method section)	DURATION (INTENSITY)	DEFINITION/s	REFERENCE
			Acute	Sub-acute	Chronic	IG	CG	Crossover						
1994	Macdonnel	USA	x	x		20	18		Clinical ratings: Barthel Index; Fugl-Meyer Lower Extremity Motor Assessment Scale; Massachusetts General Hospital Functional Ambulation Classification (MGH FAC); Electrophysiological testing: foot tap frequency; Hmax/Mmax ratio in gastrocnemius; degree of vibratory inhibition of the H reflex and Fmean/Mmax ratio	Cyclical electrical stimulation (CES) and Functional electrical stimulation (FES) with exercise or functional activities	Passive or active assisted exercises if dorsiflexion strength was MRC Grade 0-1, active assisted exercises and functional activities such as gait training, rolling, etc if dorsiflexion strength was MRC Grade 2 or higher.	4 weeks IG: CES 5 days/week, 30'-40'; FES + exercise or functional activities 3 days/week, 20' CG: self-exercise program 5 days/week, 2-3 sessions/day, 15'; exercise or functional activities 3 days/week, 20'	Conventional physical therapy	Macdonell RAL, Triggs WJ, Leikauskas J, Bourque M, Robb K, Day BJ, et al. Functional electrical stimulation to the affected lower limb and recovery after cerebral infarction. J Stroke Cerebrovasc Dis 1994;4:155–60. https://doi.org/10.1016/S1052-3057(10)80178-8.
1999	Kwakkel	Netherlands	x			IG1: 33; IG2: 31	37		Barthel Index; Functional Ambulation Categories; Action Research arm test; 10MWT; number of walking devices; Part 1 of the Nottingham health profile; a short generic version of the sickness impact profile; Frenchay activities index	IG 1: arm training + basic rehabilitation program (leg + arm rehabilitation + ADL training); IG 2: leg trainign + basic rehabilitation program (leg + arm rehabilitation + ADL training)	Immobilisation of the paretic arm and leg by means of an inflatable pressure splint + basic rehabilitation program (leg + arm rehabilitation + ADL training)	20 weeks, 5 days/week, treatment 30' + basic rehabilitation program 15' + 15' + 90'	Basic rehabilitation program; Usual care	Kwakkel G, Wagenaar RC, Twisk JW, Lankhorst GJ, Koetsier JC. Intensity of leg and arm training after primary middle-cerebral-artery stroke: a randomised trial. The Lancet 1999;354:191–6. https://doi.org/10.1016/S0140-6736(98)09477-X.

2001	Cheng	China		x		30	24		Occurrence of falls; sit-to-stand performance; including body-weight distribution; rate of rise in force; and sway in center of pressure (COP)	Conventional stroke rehabilitation program + standing postural symmetry and repetitive sit-to-stand training	Conventional stroke rehabilitation program	IG: 3 weeks, 5 days/week, 30'+20' CG: N/R	Conventional stroke rehabilitation program	Cheng PT, Wu SH, Liaw MY, Wong AM, Tang FT. Symmetrical body-weight distribution training in stroke patients and its effect on fall prevention. Arch Phys Med Rehabil 2001;82:1650–4. https://doi.org/10.1053/apmr.2001.26256.
2003	Duncan	USA	x			50	50		Ankle and knee Isometric strength; LE Fugl-Meyer; Grip Strength; Berg Balance Scale; Duration of bike exercise; 10-m gait velocity; 6-min walk distance	Excercise program designed to improve strength, balance, and endurance and to encourage more use of the affected extremity	Usual care: services as prescribed by patients' physicians	IG: 12 to 14 weeks (36 sessions; 90') CG: not specified	Usual care	Duncan P, Studenski S, Richards L, Gollub S, Lai SM, Reker D, et al. Randomized Clinical Trial of Therapeutic Exercise in Subacute Stroke. Stroke 2003;34:2173–80. https://doi.org/10.1161/01.STR.0000083699.95351.F2.
2003	Moreland	Canada	x	x		65	65		Chedoke-McMaster Stroke Assessment (CMSA) Disability Inventory; 2-minute walk test (2MWT); adverse effects; Modified Ashworth Scale (MAS); discharge living arrangements	Progressive resistance exercises + Conventional physiotherapy	Exercises with no external resistance + Conventional physiotherapy	Exercise session: 3 days/week, 30', for the duration of their stay Conventional physiotherapy: N/R	Conventional physiotherapy	Moreland JD, Goldsmith CH, Huijbregts MP, Anderson RE, Prentice DM, Brunton KB, et al. Progressive resistance strengthening exercises after stroke: a single-blind randomized controlled trial. Arch Phys Med Rehabil 2003;84:1433–40. https://doi.org/10.1016/S0003-9993(03)00360-5.
2004	Newsman	USA	x	x		10	9		Maximum voluntary isometric torque and motor unit recruitment of hemiparetic knee extensors	Eletric stimultion facilitation program + standard physical therapy care	Standard physical therapy care	4 weeks (5 to 6 day per week; 60')	Standard physical therapy care	Newsam CJ, Baker LL. Effect of an electric stimulation facilitation program on quadriceps motor unit recruitment after stroke. Arch Phys Med Rehabil 2004;85:2040–5. https://doi.org/10.1016/j.apmr.2004.02.029.
2004	Richards	Canada		x		32	31		Timeneeded to walk 5 m; 10 m; or 30 m; Fugl-Meyer assessment; timed up&go test; Barthel Index; Berg Balance Scale; gait speed;	specialized locomotor training	Conventional Physical therapy	8 weeks (5 days per week; 60')	Conventional Physical Therapy	Richards CL, Malouin F, Bravo G, Dumas F, Wood-Dauphinee S. The Role of Technology in Task-Oriented Training in Persons with Subacute Stroke: A Randomized

									mechanical power produced by the ankle plantarflexor muscles at the end of the stance phase.					Controlled Trial. Neurorehabil Neural Repair 2004;18:199–211. https://doi.org/10.1177/1545968304269397.
2005	Park	United Kingdom	x			56	60		Modified Barthel Index; National Institutes of Health Stroke Scale score; motoricity index; quality of life; EQ-VAS; Nottingham Extended ADL score; Ashworth scale for muscle spasticity; timed 10-m walk; 9-hole peg test; swallowing status; patient's blinding regarding treatment.	Real Acupuncture plus conventional multidisciplinary stroke rehabilitation	Sham Acupuncture plus conventional multidisciplinary stroke rehabilitation	2 weeks (9/12 sessions; 20' + not specified)	Conventional multidisciplinary stroke rehabilitation Routine treatment	Park J, White AR, James MA, Hemsley AG, Johnson P, Chambers J, et al. Acupuncture for Subacute Stroke Rehabilitation: A Sham-Controlled, Subject- and Assessor-Blind, Randomized Trial. Arch Intern Med 2005;165:2026. https://doi.org/10.1001/archinte.165.17.2026.
2005	Van Vliet	United Kingdom	x			60	60		Rivermead Motor Assessment; Motor Assessment Scale; ten hole peg test; 6 m walk test; Modified Ashworth Scale; Nottingham Sensory Assessment; Barthel Index; Extended Activities of Daily Living scale.	movement science based treatment	Bobath based tratment	not standard treatment length	Routine treatment	van Vliet PM. Comparison of Bobath based and movement science based treatment for stroke: a randomised controlled trial. J Neurol Neurosurg Psychiatry 2005;76:503–8. https://doi.org/10.1136/jnnp.2004.040436.
2005	Yan	Hong Kong	x			IG1: 13; IG2: 15	13		Composite spasticity scale; Maximum isometric voluntary contraction; TimeUp & Go test.	IG 1: Standard rehabilitation plus functional electric stimulation; IG 2: Standard rehabilitation plus placebo Functional Electrical Stimulation.	Standard rehabilitation	3 weeks IG 1/2 (5 days per week; 60' + 5 days per week; 30'); CG (5 days per week; 60')	Standard rehabilitation	Yan T, Hui-Chan CWY, Li LSW. Functional Electrical Stimulation Improves Motor Recovery of the Lower Extremity and Walking Ability of Subjects With First Acute Stroke: A Randomized Placebo-Controlled Trial. Stroke 2005;36:80–5. https://doi.org/10.1161/01.STR.0000149623.24906.63.
2005	Yang	Taiwan		x	x	13	12		Gait temporal-spatial variables	Conventional training programme + backward training programme	Conventional training programme	IG: 3 weeks (3 times a week, 40' + 30') CG: 3 weeks (3 times a week, 40')	Conventional training	Yang Y-R, Yen J-G, Wang R-Y, Yen L-L, Lieu F-K. Gait outcomes after additional backward walking training in patients with stroke: a

														randomized controlled trial. Clin Rehabil 2005;19:264–73. https://doi.org/10.1191/0269215505cr860oa.
2006	Tong	Hong Kong	x	x		IG1: 15; IG2: 15	20		Five meters walking speed test; Elderly Mobility Scale; Berg Balance Scale; Functional Ambulatory Category; Motricity Index led subscale; Functional Indipendence Measure; Barthel Index.	IG1: Electromechanical Gait Traniner (EGT) + regular physical therpy + multidisciplinary treatment; IG2: Electromechanical Gait Traniner with Functuonal Electrical Stimulation (EGT-FES) + regular physical therpy + multidisciplinary treatment;	CG: overground gait training + regular physical therapy + multidisciplinary treatment	4 weeks (5 day per week; 150')	Regular physical therapy; Conventional rehabilitation; Conventional PT gait training; Conventional treatment	Tong RK, Ng MF, Li LS. Effectiveness of gait training using an electromechanical gait trainer, with and without functional electric stimulation, in subacute stroke: a randomized controlled trial. Arch Phys Med Rehabil 2006;87:1298–304. https://doi.org/10.1016/j.apmr.2006.06.016.
2006	Yavuzer	Turkey	x	x		12	13		Brunnstrom stages for the lower extremity; gait kinematic parameters.	Conventional stroke rehabilitation program plus 10' of neuromuscular electric stimulation to the tibialis anterior muscle of the paretic limb	Conventional stroke rehabilitation program	4 weeks IG: (5 days per week; 120'/300' + 10'); CG (5 days per week; 120'/300')	Conventional stroke rehabilitation program	Yavuzer G, Geler-Külcü D, Sonel-Tur B, Kutlay S, Ergin S, Stam HJ. Neuromuscular electric stimulation effect on lower-extremity motor recovery and gait kinematics of patients with stroke: a randomized controlled trial. Arch Phys Med Rehabil 2006;87:536–40. https://doi.org/10.1016/j.apmr.2005.12.041.
2007	Dean	Australia	x	x		6	6		Maximum reach distance; reach movement time and peak vertical force through affected foot during reaching; peak vertical force through affected foot during standing up and 10MWT	Sitting training protocol that involved practising reaching tasks beyond arm's length + Regular physiotherapy	Sham sitting training protocol that involved practising cognitive-manipulative tasks within arm's length + Regular physiotherapy	2 weeks, 5 sessions/week, 30'	Regular physiotherapy	Dean CM, Channon EF, Hall JM. Sitting training early after stroke improves sitting ability and quality and carries over to standing up but not to walking: a randomised trial. Aust J Physiother 2007;53:97–102.
2007	Sutbeyaz S	Turkey		x		20	20		Brunnstrom stages of motor recovery; Modified Asworth Scale; Functional Ambulation Categories; motor	Mirror therapy + conventional stroke rehabilitation	Conventional stroke rehabilitation	4 weeks (5 days a week; 120-300')	Conventional stroke rehabilitation	Sütbeyaz S, Yavuzer G, Sezer N, Koseoglu BF. Mirror therapy enhances lower-extremity motor recovery and motor functioning after stroke:

									items of Functional Indipendence Measure					a randomized controlled trial. Arch Phys Med Rehabil 2007;88:555–9. https://doi.org/10.1016/j.apmr.2007.02.034.
2007	Tseng	Taiwan			x	IG1: 22; IG2: 21	22		FIM-ADL subscale; ROM; 0-2 pain scale; Chinese version of Geriatric Depression Scale – Short Form (GDS-15)	IG1: registered nurse supervised ROM exercises; IG2: registrered nurse assististed ROM exercises	Usual care	IG: 4 weeks, 6 days/week, 2 sessions/day, 10-20' CG: N/R	Usual care	Tseng C-N, Chen CC-H, Wu S-C, Lin L-C. Effects of a range-of-motion exercise programme. J Adv Nurs 2007;57:181–91. https://doi.org/10.1111/j.1365-2648.2006.04078.x.
2007	Wang	China		x		22	22		Brunnstrom stage; Barthel Index	Upper limbs' encircling motion (ULEM) therapy + conventional physical therapy	Conventional physical therapy	IG: 4 weeks, 5 days/week, 5 sessions/day (ULEM training) + 60' (conventional physical therapy) CG: 4 weeks, 5 days/week, 60'	Conventional physical therapy; Traditional rehabilitation; Traditional physical therapy	Wang T, Wang X, Wang H, He X, Su J, Zhu Y, et al. Effects of ULEM apparatus on motor function of patients with stroke. Brain Inj 2007;21:1203–8. https://doi.org/10.1080/02699050701633064.
2007	Yavuzer	Turkey		x		15	15		Brunnstrom stages for the lower extremity; gait kinematics parameters	Conventional rehabilitation program + Sensory-amplitude electric stimulation (SES)	Conventional rehabilitation program + sham SES	4 weeks, 5 days/week, 30' (SES) + 120'-300' (conventional rehabilitation program)	Conventional stroke rehabilitation program	Yavuzer G, Oken O, Atay MB, Stam HJ. Effect of sensory-amplitude electric stimulation on motor recovery and gait kinematics after stroke: a randomized controlled study. Arch Phys Med Rehabil 2007;88:710–4. https://doi.org/10.1016/j.apmr.2007.02.030.
2007	Yen	Taiwan			x	7	7		Berg Balance Scale (BBS); gait parameters (walking speed; cadence; and step length); corticomotor activity (motor threshold; map size; location of the amplitude-weighted center of gravity of the motor map)	Additional body weight–supported treadmill training (BWSTT) + general physical therapy	General physical therapy	IG: 4 weeks, 3 days/week, 30' (additional gait training) + 2-5 days/week, 50' (general physical therapy) CG: 4 weeks, 2-5 days/week, 50' (general physical therapy)	General physical therapy	Yen C-L, Wang R-Y, Liao K-K, Huang C-C, Yang Y-R. Gait Training-Induced Change in Corticomotor Excitability in Patients With Chronic Stroke. Neurorehabil Neural Repair 2008;22:22–30. https://doi.org/10.1177/1545968307301875.
2008	Eser	Turkey		x	x	25	25		Brunnstrom stages; Rivermead Mobility Index; Functional	conventional stroke rehabilitation program plus balance training using	conventional stroke rehabilitation program	IG 8 weeks (5 days per week; 120'/300') + 3 weeks (5 days per week;	Conventional stroke rehabilitation	Eser F, Yavuzer G, Karakus D, Karaoglan B. The effect of balance training on motor

									Independence Measure.	the Nor-Am Target Balance Training System		15'); CG 8 weeks (5 days per week; 120'/300')		recovery and ambulation after stroke: a randomized controlled trial. Eur J Phys Rehabil Med 2008;44:19–25.
2008	Ferrante	Italy	x	x		10	10		Maximum isometric voluntary contraction (MVC) of quadriceps; walking and sit-to-stand ability; Motricity Index (MI); Upright Motor Control Test (UMC); Trunk Control Test (TCT)	Cycling induced by functional electrical stimulation + standard rehabilitation	Standard rehabilitation	4 weeks, 180'	Standard rehabilitation	Ferrante S, Pedrocchi A, Ferrigno G, Molteni F. Cycling induced by functional electrical stimulation improves the muscular strength and the motor control of individuals with post-acute stroke. Eur J Phys Rehabil Med 2008;44:159–67.
2008	Gok	Turkey			x	15	15		Kinesthetic Ability Trainer as balance index; Functional Independence Measure; Fugl-Meyer Stroke Assessment Instrument.	Conventional rehabilitation programme plus balance training programme	Conventional rehabilitation programme	4 weeks IG: (5 days per week; 120'/180' + 20'); CG (5 days per week; 120'/180')	Conventional rehabilitation programme	Gok H, Geler-Kulcu D, Alptekin N, Dincer G. Efficacy of treatment with a kinaesthetic ability training device on balance and mobility after stroke: a randomized controlled study. Clin Rehabil 2008;22:922–30. https://doi.org/10.1177/0269215508090673.
2009	Mesci	Turkey		x	x	20	20		Ankle passive dorsiflexion range of motion; Modified Ashworth Scale; Brunnstrom Stage; Functional Independence Measurement; Rivermead Motor Assessment Scale; Functional Ambulation Categories.	neuromuscular electrical stimulation applied on the foot dorsiflexors plus conventional exercise program	conventional exercise program	IG 4 weeks (5 days per week; 20' + not specified) CG not specified	Conventional exercise program; Traditional treatment exercise program; Conventional rehabilitation program	Mesci N, Ozdemir F, Kabayel DD, Tokuc B. The effects of neuromuscular electrical stimulation on clinical improvement in hemiplegic lower extremity rehabilitation in chronic stroke: A single-blind, randomised, controlled trial. Disabil Rehabil 2009;31:2047–54. https://doi.org/10.3109/09638280902893626.
2009	Yan	China	x			IG1: 21; IG2: 21	20		Composite Spasticity Scale (CSS); Maximum isometric voluntary contraction (MIVC) of the anklen dorsiflexor and plantarflexor	IG1: Transcutaneous electrical stimulation + Standard rehabilitation; IG2: Placebo stimulation + Standard rehabilitation	Standard rehabilitation	IG1 / IG2: 3 weeks, 5 days/week, 60' (stimulation) + 60' (standard rehabilitation) CG2: 3 weeks, 5 days/week, 60' (standard rehabilitation)	Standard rehabilitation	Yan T, Hui-Chan CWY. Transcutaneous electrical stimulation on acupuncture points improves muscle function in subjects after acute stroke: a randomized controlled

									muscles; Timed "Up & Go" (TUG) test					trial. J Rehabil Med 2009;41:312–6. https://doi.org/10.2340/16501977-0325.
2010	Cooke	United Kingdom	x	x		IG1: 35; IG2: 36	38		Walking speed; ability to walk at least at 0.8 m/s; Dynami torque of the paretic knee; Rivermead mobility Index; Temporo-spatial gait parameters; EuroQuol.	IG 1: Conventional Physiotherapy soft tissue mobilization, facilitation of muscle activity, facilitation of coordinated multijoint movement, tactile and proprioceptive input, resistive exercise, and functional retraining, repeated for 2 sessions per day; IG 2: Functional strenght training repetitive, progressive resistive exercise during goal-directed functional activity plus conventional physiotherapy	Conventional Physiotherapy soft tissue mobilization, facilitation of muscle activity, facilitation of coordinated multijoint movement, tactile and proprioceptive input, resistive exercise, and functional retraining	IG 1/2 Additional intervention: 6 weeks (4 days per week; 60'); Conventional physiotherapy time not specified	Conventional Physiotherapy; Routine conventional physiotherapy	Cooke EV, Tallis RC, Clark A, Pomeroy VM. Efficacy of functional strength training on restoration of lower-limb motor function early after stroke: phase I randomized controlled trial. Neurorehabil Neural Repair 2010;24:88–96. https://doi.org/10.1177/1545968309343216.
2010	Goljar	Slovenia		x	x	22	22		Functional Independence Measure; Berg Balance Scale; one-leg standing; Timed Up and Go Test; 10 m walk test.	physiotherapy with balance trainer program	conventional physiotherapy program	4 weeks (5 days per week; 45')	Conventional physiotherapy program	Goljar N, Burger H, Rudolf M, Stanonik I. Improving balance in subacute stroke patients: a randomized controlled study. Int J Rehabil Res 2010;33:205–10. https://doi.org/10.1097/MRR.0b013e328333de61.
2010	Ivey	USA			x	39	41		Leg Blood Flow Test; walking velocity; Peak aerobic testing.	treadmill testing protocol with target aerobic intensity	physical therapy exercises common to stroke	24 weeks (3 sessions per week; 40')	Physical therapy common exercises	Ivey FM, Hafer-Macko CE, Ryan AS, Macko RF. Impaired Leg Vasodilatory Function After Stroke: Adaptations With Treadmill Exercise Training. Stroke 2010;41:2913–7. https://doi.org/10.1161/STROKEAHA.110.599977.
2010	Jonsdottir	Italy			x	10	10		Ankle power peak; Gait velocity.	Biofeedback signal driven by the EMG of	Traditional physical therapy	7 weeks (3 sessions per week; 45')	Traditional physical therapy; Usual	Jonsdottir J, Cattaneo D, Recalcati M, Regola A, Rabuffetti M, Ferrarin M,

										the gastrocnemius lateralis during gait.			rehabilitation care; Conventional therapy	et al. Task-Oriented Biofeedback to Improve Gait in Individuals With Chronic Stroke: Motor Learning Approach. Neurorehabil Neural Repair 2010;24:478–85. https://doi.org/10.1177/1545968309355986.
2010	Paoloni	Italy			x	22	22		kinematic data of gait; EMG of Tibialis Anterior and Gastrocnemius Medialis.	General physical therapy plus Segmental muscle vibration	General physical therapy	4 weeks IG (3 sessions per week; 50' + 30') CG (3 sessions per week; 50')	General physical therapy	Paoloni M, Mangone M, Scettri P, Procaccianti R, Cometa A, Santilli V. Segmental muscle vibration improves walking in chronic stroke patients with foot drop: a randomized controlled trial. Neurorehabil Neural Repair 2010;24:254–62. https://doi.org/10.1177/1545968309349940.
2010	Tihanyi	Hungary	x	x		10	10		Knee extensors: maximum isometric torque; maximum rate of torque development; maximum eccentric torque; mechanical work ; Knee extensors myoelectrical activity	Whole body vibration + conventional rehabilitation	Conventional rehabilitation	IG: 4 weeks (3 times per week); CG: not specified	Conventional rehabilitation	Tihanyi J, Di Giminiani R, Tihanyi T, Gyulai G, Trzaskoma L, Horváth M. Low resonance frequency vibration affects strength of paretic and non-paretic leg differently in patients with stroke. Acta Physiol Hung 2010;97:172–82. https://doi.org/10.1556/APhysiol.97.2010.2.3.
2010	Tung	Taiwan		x	x	16	16		Balance Master System; Berg Balance Scale; extensor strength of lower extremity.	General physical therapy plus sit-to-stand training programme	General physical therapy	4 weeks IG (3 sessions per week; 30' + 15'); CG (3 sessions per week; 30')	General physical therapy	Tung F-L, Yang Y-R, Lee C-C, Wang R-Y. Balance outcomes after additional sit-to-stand training in subjects with stroke: a randomized controlled trial. Clin Rehabil 2010;24:533–42. https://doi.org/10.1177/0269215509360751.
2011	Galvin	Ireland	x			20	20		Modified LE Fugl-Meyer	FAME (family-mediated exercises) + Routine physiotherapy	Routine physioptherapy	8 weeks (IG: 35 min ; CG: not specifeid)	Routine therapy; Routine physiotherapy	Galvin R, Cusack T, O'Grady E, Murphy TB, Stokes E. Family-Mediated Exercise Intervention (FAME): Evaluation of a Novel

														Form of Exercise Delivery After Stroke. Stroke 2011;42:681–6. https://doi.org/10.1161/STROKEAHA.110.594689.
2011	Hesse	Germany		x		25	25		Rivermead Mobility Index (RMI); Rivermead Motor Assessment - section leg and trunk; Rivermead Motor Assessment - arm section; walking velocity; stair climbing velocity; Timed up and go test (TUG); modified Ashworth Scale; Rivermead Activities of Daily Living scales; adverse events; falls	Intermittent high-intensity physiotherapy programme at home	Regular physiotherapy programme for home-based chronic stroke patients	IG: three two-month blocks, 4 days/week, 30'-45' (96 sessions) CG: 12 months, 2 days/week, 30'-45' (104 sessions)	Regular physiotherapy programme	Hesse S, Welz A, Werner C, Quentin B, Wissel J. Comparison of an intermittent high-intensity vs continuous low-intensity physiotherapy service over 12 months in community-dwelling people with stroke: a randomized trial. Clin Rehabil 2011;25:146–56. https://doi.org/10.1177/0269215510382148.
2011	Kim	Republic of Korea			x	24	20		10MWT; Timed up and go test (TUG); Berg Balance Scale (BBS); Balance Performance Monitor (BPM)	Lower extremity muscle strengthening + General physical therapy	Treadmill walking + General physical therapy	6 weeks, 3 days/week, 30' + 5 days/week 30' (NDT) + 20' (FES)	General physical therapy	Kim CS, Gong W, Kim SG. The Effects of Lower Extremitiy Muscle Strengthening Exercise and Treadmill Walking Exercise on the Gait and Balance of Stroke Patients. J Phys Ther Sci 2011;23:405–8. https://doi.org/10.1589/jpts.23.405.
2011	Sungkarat	Thailand		x	x	20	20		Gait speed; step length asymmetry ratio and single support time asymmetry ratio; Berg Balance Scale (BBS); Timed up and go test (TUG)	Conventional stroke rehabilitation programme with the use of the Insole Shoe Wedge and Sensors set-up	Conventional stroke rehabilitation programme	3 weeks, 5 days/week, 60'	Conventional gait training programme; Conventional stroke rehabilitation programme	Sungkarat S, Fisher BE, Kovindha A. Efficacy of an insole shoe wedge and augmented pressure sensor for gait training in individuals with stroke: a randomized controlled trial. Clin Rehabil 2011;25:360–9. https://doi.org/10.1177/0269215510386125.
2011	Varoqui	France	x	x		IG1: 8; IG2: 8	8		Lowe limbs muscle strength; modified Ashworth scale; Postural Assessment Scale for Stroke; Berg Balance Scale; Functional	IG 1: Biofeedback training with feedback from the non affected side plus daily physical therapy; IG 2: Biofeedback training	Stand up task plus daily physical therapy	4 weeks (number of sessions not specified; 15' + 60')	Daily physical therapy	Varoqui D, Froger J, Pélissier J-Y, Bardy BG. Effect of coordination biofeedback on (re)learning preferred postural patterns in post-stroke patients. Motor

									Ambulatory Category; Functional Independence Measure.	with feedback from the affected side plus daily physical therapy				Control 2011;15:187–205. https://doi.org/10.1123/mcj.15.2.187.
2012	Batchelor	Australia		x		71	85		Fall rates and the proportion of fallers in the 12 months after baseline assessment; rate of injurious falls; Sit-ToStand test; Gait Speed; Step Test; Human Activity Profile; Functional Indipendence Measure; Falls Efficacy Scale; Falls Risk for Older People; Baking Tray Task/Star Cancellation Test; Visual Field Cnfrontation Test; Abbreviated Mental Test Score.	Usual care plus a multifactorial, individually tailored falls prevention program	Usual care	Not specified	Usual care	Batchelor FA, Hill KD, Mackintosh SF, Said CM, Whitehead CH. Effects of a Multifactorial Falls Prevention Program for People With Stroke Returning Home After Rehabilitation: A Randomized Controlled Trial. Arch Phys Med Rehabil 2012;93:1648–55. https://doi.org/10.1016/j.apmr.2012.03.031.
2012	Chang	Republic of Korea	x			21	20		Fugl-Meyer Assessment scale - lower extremiti (FMA-L); leg score of the Motricity Index (MI-L); Functional Ambulation Category (FAC); cardiopulmonary fitness measures (peakVO2; oxygen pulse; peak heart rate; blood pressure; RPE; minute ventilation)	Robot-assisted gait training + Conventional physical therapy	Conventional physical therapy	IG: 2 weeks, 5 days/week, 2 sessions/day, 40' (BWSTT) + 60' (conventional physical therapy) CG: 2 weeks, 5 days/week, 2 sessions/day, 100' (conventional physical therapy)	Conventional physical therapy	Chang WH, Kim MS, Huh JP, Lee PKW, Kim Y-H. Effects of robot-assisted gait training on cardiopulmonary fitness in subacute stroke patients: a randomized controlled study. Neurorehabil Neural Repair 2012;26:318–24. https://doi.org/10.1177/1545968311408916.
2012	Chumbler	USA	x	x	x	25	23		Telephone Version of the Functional Independence Measure; Overall Function Component of the Late-Life Function and Disability Instrument.	STeleR Intervention	usual care	12 weeks IG (3 home visits; 60' + daily messaging + 5 telephone calls) CG not specified	Usual care	Chumbler NR, Quigley P, Li X, Morey M, Rose D, Sanford J, et al. Effects of Telerehabilitation on Physical Function and Disability for Stroke Patients: A Randomized, Controlled Trial. Stroke 2012;43:2168–74. https://doi.org/10.1161/STROKEAHA.111.646943.

2012	Globas	Germany			x	20	18		VO2peak; 6MWT; gait velocity; Berg Balance Scale; 5 chair-rise test; Rivermead Mobility Index (RMI); SF-12	Progressive graded, high-intensity aerobic treadmill exercise	Conventional care physiotherapy	IG: 3 months, 3 days/week, 30'-50' CG: 3 months, 1-3 days/week, 60'	Conventional care physiotherapy	Globas C, Becker C, Cerny J, Lam JM, Lindemann U, Forrester LW, et al. Chronic Stroke Survivors Benefit From High-Intensity Aerobic Treadmill Exercise: A Randomized Control Trial. Neurorehabil Neural Repair 2012;26:85–95. https://doi.org/10.1177/1545968311418675.
2012	Mohapatra	USA	x	x		5	6		Weight bearing on afected side; Gait velocity; Berg Balance Scale; Fugl-Meyer Assessment	Complelled body weight shift + Conventional physiotherapy	Conventional physical therapy	2 weeks (6 times per week; 90')	Conventional physical therapy	Mohapatra S, Eviota AC, Ringquist KL, Muthukrishnan SR, Aruin AS. Compelled Body Weight Shift Technique to Facilitate Rehabilitation of Individuals with Acute Stroke. ISRN Rehabil 2012;2012:1–7. https://doi.org/10.5402/2012/328018.
2013	Hsu	Taiwan		x	x	17	17		Low Extremity subscale of the Stroke Rehabilitation Assessment of Movement; Mobility subscale od STREAM; Functional Ashworth Scale.	Regular rehabilitation + Thermal stimulation (noxius)	Regular rehabilitation + Thermal stimulation (innocus)	8 weeks (3 day per week; 120' + 30')	Regular rehabilitation	Hsu H-W, Lee C-L, Hsu M-J, Wu H-C, Lin R, Hsieh C-L, et al. Effects of noxious versus innocuous thermal stimulation on lower extremity motor recovery 3 months after stroke. Arch Phys Med Rehabil 2013;94:633–41. https://doi.org/10.1016/j.apmr.2012.11.021.
2013	MacKay-Lyons	Canada	x	x		24	26		VO2peak; Heart rate; respiratory exchange ratio; and RPE0-10 at peak exercise; 6 minute walking test distance; Berg Balance Scale; Chedoke-McMaster Stages of Recovery.	Body-weight-supported treadmill training	usual care	6 weeks (5 days per week; 60' + 3 days per week; 60')	Usual care	Mackay-Lyons M, McDonald A, Matheson J, Eskes G, Klus M-A. Dual effects of body-weight supported treadmill training on cardiovascular fitness and walking ability early after stroke: a randomized controlled trial. Neurorehabil Neural Repair 2013;27:644–53.

														https://doi.org/10.1177/1545968313484809.
2013	Rao	USA	x			14	14		Fugl-Meyer Balance test (FM-B); Functional Independene Measure test for gait (FIMG); Fugl-Meyer lower extremity assessment (FM-LE)	Weight support balance therapy with visual feedback + Physical therapy + Occupational therapy + Speech therapy + Neuropsychology	Balance training + Physical therapy + Occupational therapy + Speech therapy + Neuropsychology	Functional training: 3 sessions Physical therapy: 2 weeks, 6 days/week, 60' (weekdays) + 30' (Saturdays) Occupational therapy + Speech therapy + Neuropsychology: N/R	Conventional treatment	Rao N, Zielke D, Keller S, Burns M, Sharma A, Krieger R, et al. Pregait balance rehabilitation in acute stroke patients. Int J Rehabil Res 2013;36:112–7. https://doi.org/10.1097/MRR.0b013e328359a2fa.
2013	Sekhar	India	N/A	N/A	N/A	20	20		Peak torque of knee extensors on paretic leg; Berg Balance Scale	Isokinetic strength traning + Balance exercises	Conventional physiotherapy	6 weeks	Conventional physiotherapy	Sekhar PKC, Madhavi K, Srikumari V. Efficacy of Isokinetic Strength Training and Balance Exercises on Lower Limb Muscles in Subjects with Stroke. Indian J Physiother Occup Ther - Int J 2013;7:112. https://doi.org/10.5958/j.0973-5674.7.4.132.
2013	Sheffler	USA		x	x	54	56		Fugl-Meyer Assessment for Lower extremity; modified Emory Functional Ambulation Profile; Stroke Specific Quality of Life score.	Standard Physical Therapy while wearing a Peroneal nerve stimulator, the subjects were trained to use the device in a Functional Training phase after these a Post-Functional Training phase was held.	Standard Physical Therapy	12 weeks IG: 5 weeks (2 sessions per week; 60'); 7 weeks (3 sessions per week; 60'). Standard Physical Therapy time not specified	Standard Physical Therapy	Sheffler LR, Taylor PN, Gunzler DD, Buurke JH, Ijzerman MJ, Chae J. Randomized controlled trial of surface peroneal nerve stimulation for motor relearning in lower limb hemiparesis. Arch Phys Med Rehabil 2013;94:1007–14. https://doi.org/10.1016/j.apmr.2013.01.024.
2013	Zhang	China	x			46	23		Fugl-Meyer Assessment (FMA); National Institutes of Health Stroke Scale (NIHSS); Barthel index (BI); modified Rankin Scale (mRS)	Integrated Rehabilitation Techniques of Traditional Chinese Medicine (IRT-TCM) - massage and acupuncture	Conventional rehabilitation techniques	IG: 30' acupuncture + 30' massage/day until discharge CG: 60'/day until discharge	Conventional rehabilitation techniques	Zhang Y, Jin H, Ma D, Fu Y, Xie Y, Li Z, et al. Efficacy of Integrated Rehabilitation Techniques of Traditional Chinese Medicine for Ischemic Stroke: A Randomized Controlled Trial. Am J Chin Med 2013;41:971–81. https://doi.org/10.1142/S0192415X13500651.
2014	Cha	Republic of Korea			x	10	10		Grip strength; Fugl-Meyer assessment.	Basic training for functional improvement plus	Basic training for functional improvement	4 weeks IG (5 sessions per week; 30' + 20'); CG (5 sessions per week; 30')	Basic training	Cha H-K, Ji S-G, Kim M-K, Chang J-S. Effect of Transcranial Direct Current Stimulation of

										transcranial direct current stimulation				Function in Patients with Stroke. J Phys Ther Sci 2014;26:363–5. https://doi.org/10.1589/jpts.26.363.
2014	Chen	Taiwan			x	15	16		Turning performance ; Speed and temporal-spatial gait parameters ; Berg Balance Scale ; Limits of Stability Test ; Lower extremity strength	Turning-based treadmill training	Regular treadmill + general exercise program	IG: 4 weeks (12 session; 30') CG: 4 weeks (12 sessions; 30' + 10')	General exercise program	Chen I-H, Yang Y-R, Chan R-C, Wang R-Y. Turning-Based Treadmill Training Improves Turning Performance and Gait Symmetry After Stroke. Neurorehabil Neural Repair 2014;28:45–55. https://doi.org/10.1177/1545968313497102.
2014	De Nunzio	Italy	x			18	19		Standing Balance Score; Unified Balance Scale; Functional Independence Measure; Satisfaction Score; Center of Pressure analysis.	Traditional rehabilitation techniques plus Weight-shifting exercises	Traditional rehabilitation techniques plus physiotherapy	2 weeks (6 days per week; 60' + 30')	Traditional rehabilitation techniques	De Nunzio AM, Zucchella C, Spicciato F, Tortola P, Vecchione C, Pierelli F, et al. Biofeedback rehabilitation of posture and weightbearing distribution in stroke: a center of foot pressure analysis. Funct Neurol 2014;29:127–34.
2014	Dragin	Serbia	x	x		11	11		Barthel Index (BI); Fugl-Meyer score for the lower extremities (FM-LE); Berg Balance Test (BBS); gait speed	Body postural support gait training + Conventional physical therapy + Occupational therapy + Speech therapy	Conventioanl gait assistance training + Conventional physical therapy + Occupational therapy + Speech therapy	4 weeks	Conventional physical therapy	Dragin AS, Konstantinović LM, Veg A, Schwirtlich LB. Gait training of poststroke patients assisted by the Walkaround (body postural support). Int J Rehabil Res Int Z Rehabil Rev Int Rech Readaptation 2014;37:22–8. https://doi.org/10.1097/MRR.0b013e328363ba30.
2014	Hung	Taiwan			x	15	15		Static standing balance; Forward reach test; timed up&go test; Falls Efficacy Scale-International questionnaire; Physical Activity Enjoyment Scale;	Routine rehabilitation training plus Wii Fit training	Routine rehabilitation training plus conventional weight-shifting training	12 weeks (not pecified + 30')	Routine rehabilitation training	Hung J-W, Chou C-X, Hsieh Y-W, Wu W-C, Yu M-Y, Chen P-C, et al. Randomized Comparison Trial of Balance Training by Using Exergaming and Conventional Weight-Shift Therapy in Patients With Chronic Stroke. Arch Phys Med Rehabil 2014;95:1629–37.

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2014	Ji	Republic of Korea		x		17	17		Temporospatial gait characteristics: step length; stride length; rate of swing phase; rate of stance phase; velocity; cadence; step length	Mirror therapy + conventional rehabilitation therapy	Sham therapy + conventional rehabilitation therapy	4 weeks (5 day per week: 15' + 30')	Conventional rehabilitation therapy	Ji SG, Kim MK. The effects of mirror therapy on the gait of subacute stroke patients: a randomized controlled trial. Clin Rehabil 2015;29:348–54. https://doi.org/10.1177/0269215514542356.
2014	McEwen	Canada	x	x		33	41		Timed Up and Go test (TUG); 2MWT; Chedoke McMaster Stroke Assessment Scale Leg domain	VR exercises that challenged balance, performed standing + Regular inpatient rehabilitation therapy	VR exercises not challenging balance, performed sitting + Regular inpatient rehabilitation therapy	3 weeks, 10-12 sessions, 20' (VR exercises) + N/R (regular inpatioent rehabilitation therapy sessions)	Standard stroke rehabilitation therapy; Conventional inpatient stroke rehabilitation program; Regular inpatient rehabilitation therapy	McEwen D, Taillon-Hobson A, Bilodeau M, Sveistrup H, Finestone H. Virtual reality exercise improves mobility after stroke: an inpatient randomized controlled trial. Stroke 2014;45:1853–5. https://doi.org/10.1161/STROKEAHA.114.005362.
2014	Suh	Republic of Korea			x	21	21		Modified Ashworth Scale; Functional Reach Test; Berg Balance Scale; Timed Up and Go Test; 10m Walk Test	Interferential current therapy (ICT) + standard rehabilitation program	Placebo-ICT + standard rehabilitation program	1 session 60' + 30'	Standard rehabilitation program	Suh HR, Han HC, Cho H. Immediate therapeutic effect of interferential current therapy on spasticity, balance, and gait function in chronic stroke patients: a randomized control trial. Clin Rehabil 2014;28:885–91. https://doi.org/10.1177/0269215514523798.
2014	van Nunen	Netherlands		x		16	14		walking speed at the 10 m timed walk test; Functional Ambulation Category; Berg Balance Scale; Brunnstrom-Fugl-Meyer; Rivermead Mobility Index; Timed Up and Go; maximal voluntary isometric torque of the knee extensors and flexors; Short Form 36.	Lokomat plus conventional therapy	Conventional therapy	8 weeks (210' per week)	Conventional therapy Conventional physical therapy	van Nunen MPM, Gerrits KHL, Konijnenbelt M, Janssen TWJ, de Haan A. Recovery of walking ability using a robotic device in subacute stroke patients: a randomized controlled study. Disabil Rehabil Assist Technol 2015;10:141–8. https://doi.org/10.3109/17483107.2013.873489.

2014	Yang	Taiwan		x	x			31 (16/15)	Lower limb subscale of the Fugl-Meyer assessment (LE-FMA); 6-minute walk test (6MWT); 10-meter walk test (10MWT); modified Ashworth scale (MAS); bike parameters including symmetry of bilateral exertion; performance; and resistance	4 weeks biofeedback cycling + conventional rehabilitation followed by 4 weeks conventional rehabilitation only	4 weeks conventional rehabilitation only followed by 4 weeks biofeedback cycling + conventional rehabilitation	4 weeks, 5 days/week, 30' (cycling training) + 120' (conventional rehabilitation) + 4 weeks, 5 days/week, 120' (conventional rehabilitation)	Conventional rehabilitation; Regular conventional outpatient rehabilitation	Yang H-C, Lee C-L, Lin R, Hsu M-J, Chen C-H, Lin J-H, et al. Effect of biofeedback cycling training on functional recovery and walking ability of lower extremity in patients with stroke. Kaohsiung J Med Sci 2014;30:35–42. https://doi.org/10.1016/j.kjms.2013.07.006.
2014	You	China	x	x		19	18		Ankle plantarflexor Composite Spasticity Scale; lower extremity Fugl-Myer Assessment; postural assessment scale for stroke; Berg Balance Scale; modified Barthel Index.	standard rehabilitation plus Functional electrical stimulation	standard rehabilitation	3 weeks IG (5 days per week; 60' + 30'); CG (5 days per week; 60')	Standard rehabilitation	You G, Liang H, Yan T. Functional electrical stimulation early after stroke improves lower limb motor function and ability in activities of daily living. NeuroRehabilitation 2014;35:381–9. https://doi.org/10.3233/NRE-141129.
2015	Chang	Republic of Korea	x			12	12		Motor Evocated Potentials of the Tibialis Aterior; gait analysis parameters; lower limb subscale of Fugl-Meyer Assessment; lower limb Motricity Index; Functional Ambulatory Category; Berg Balance Scale.	Anodal Transcranial direct current stimulation while the patient was receiving conventional physical therapy	Sham Transcranial direct current stimulation while the patient was receiving conventional physical therapy	2 weeks (6 sessions per week; 150'/60'	Conventional physical therapy	Chang MC, Kim DY, Park DH. Enhancement of Cortical Excitability and Lower Limb Motor Function in Patients With Stroke by Transcranial Direct Current Stimulation. Brain Stimulat 2015;8:561–6. https://doi.org/10.1016/j.brs.2015.01.411.
2015	da Silva Ribeiro	Brazil			x	15	15		SF-36; Fugl–Meyer (FM) scales	Virtual rehabilitation with Nintendo Wii	Conventional physical therapy	8 weeks, 2 sessions/week, 60'	Conventional physical therapy	da Silva Ribeiro NM, Ferraz DD, Pedreira É, Pinheiro Í, da Silva Pinto AC, Neto MG, et al. Virtual rehabilitation via Nintendo Wii® and conventional physical therapy effectively treat post-stroke hemiparetic patients. Top Stroke Rehabil 2015;22:299–305. https://doi.org/10.1179/

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2015	Ding	China	x			IG1: 35; IG2: 35	33		Clinic Spasticity Influx (CSI); Fugl-Meyer Assessment (FMA); Berg Balance Scale (BBS); Functional Independence Measure (FIM)	IG1 (Treatment group): Static or dynamic ankle foot brace + color Doppler ultrasound-guided BTX-A injection + conventional rehabilitation training; IG2 (Observation group): color Doppler ultrasound-guided BTX-A injection + conventional rehabilitation training	Conventional rehabilitation training	N/R	Conventional therapy; Conventional rehabilitation training	Ding X-D, Zhang G-B, Chen H-X, Wang W, Song J-H, Fu D-G. Color Doppler ultrasound-guided botulinum toxin type a injection combined with an ankle foot brace for treating lower limb spasticity after a stroke. Eur Rev Med Pharmacol Sci 2015;19:406–11.
2015	Fujino	Japan	x			15	15		Stroke Impairment Assessment Set; Trunk Control Test ; Postural alignment (Head orientation; body axis)	Sitting balance training (on tilted platform) + conventional therapy	Sitting balance training + conventional therapy	1 week (6 session per week ; 60')	Conventional therapy; Conventional physiotherapy	Fujino Y, Amimoto K, Fukata K, Ishihara S, Makita S, Takahashi H. Does training sitting balance on a platform tilted 10° to the weak side improve trunk control in the acute phase after stroke? A randomized, controlled trial. Top Stroke Rehabil 2016;23:43–9. https://doi.org/10.1179/1945511915Y.0000000010.
2015	Gürcan	Turkey	x	x	x	19	13		Brunnstrom stages of motor recovery; Modified Asworth Scale; Functional Ambulation Categories; ROM; Functional Indipendence Measure Electriphysiological evaluations: motor conduction of the tibial and peroneal nerve; and sensory conduction of the sural nerve in the lower extremity. H-reflex and M-	Conventional treatments methods + electrical stimulation of bilateral gastrocnemius–soleus muscle	Conventional treatments methods	3 weeks (IG: 5 session/day per week 20' ; CG: not specified)	Conventional rehabilitation program; Conventional treatment methods	Gurcan A, Selcuk B, Onder B, Akyuz M, Akbal Yavuz A. Evaluation of Clinical and Electrophysiological Effects of Electrical Stimulation on Spasticity of Plantar Flexor Muscles in Patients with Stroke. Türkiye Fiz Tip Ve Rehabil Derg 2015;61:307–13. https://doi.org/10.5152/tftrd.2015.38981.

									response of the bilateral gastrocnemius–soleusmuscle and the tibial nerve F-wave response recorded from the abductor halluces longus muscle					
2015	Hwang	Republic of Korea			x	16	16		Timed Up and Go (TUG) test; Berg Balance Scale (BBS); 10-meter walk test (10MWT); ultrasound measurement of structure of the tibialis anterior	Treadmill training combined with tilt sensor FES (TTSF) + conventional physical therapy	Treadmill training combined with placebo tilt sensor FES (TPTSF) + conventional physical therapy	Treadmill training: 4 weeks, 1 sessions/day, 30' Conventional physical therapy: 4 weeks, 2 sessions/day, 30'	Conventional physical therapy	Hwang D-Y, Lee H-J, Lee G-C, Lee S-M. Treadmill training with tilt sensor functional electrical stimulation for improving balance, gait, and muscle architecture of tibialis anterior of survivors with chronic stroke: A randomized controlled trial. Technol Health Care 2015;23:443–52. https://doi.org/10.3233/THC-150903.
2015	Kawakami	Japan	x	x		IG1: 16; IG2: 19; IG3: 15; IG4: 9	8		Stroke Impairment Assessment Set (Hip-Flexion; Knee-Extension; Foot-Pat scores)	IG1: mirror therapy (MT); IG2: integrated volitional-control electrical stimulation (IVES); IG3: therapeutical electrical stimulation (TES); IG4: repetitive facilitative exercises (RFEs)	Standard training	4 weeks (IG: 20' + 40' standard training; CG: 60')	Standard training	Kawakami K, Miyasaka H, Nonoyama S, Hayashi K, Tonogai Y, Tanino G, et al. Randomized controlled comparative study on effect of training to improve lower limb motor paralysis in convalescent patients with post-stroke hemiplegia. J Phys Ther Sci 2015;27:2947–50. https://doi.org/10.1589/jpts.27.2947.
2015	Kim (a)	Republic of Korea		x	x	15	15		Gait performance; Lower Extremity muscle strength.	IG 1: Functional training plus one-leg standing training on a tilt table; IG 2: Functional training plus progressive task-oriented training on a tilt table.	Functional training plus training on a tilt table	3 weeks (5 sessions per week; 30' + 20')	Functional training Routine therapy	Kim C-Y, Lee J-S, Kim H-D, Kim J, Lee I-H. Lower extremity muscle activation and function in progressive task-oriented training on the supplementary tilt table during stepping-like movements in patients with acute stroke hemiparesis. J Electromyogr Kinesiol 2015;25:522–30.

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2015	Kim (b)	Republic of Korea	x			IG1: 13; IG2: 13	13		sEMG analysis (muscle activation) ; pressure distirbution of plantar surface ;Barthel Index; National Institute Health Stroke Scale; LE Fugl-Meyer Scale; a 6-grade scale for hemiparesis gravity assessment	IG1: tilt-table + routine therapy; IG2 task-oriented training on tilt-table + routine therapy	Routine therapy	3 weeks (5 time per week; IG: 30' + 20'; CG: 30')	Routine therapy	Kim C-Y, Lee J-S, Kim H-D, Kim J-S. The effect of progressive task-oriented training on a supplementary tilt table on lower extremity muscle strength and gait recovery in patients with hemiplegic stroke. Gait Posture 2015;41:425–30. https://doi.org/10.1016/j.gaitpost.2014.11.004.
2015	Lee	Republic of Korea			x	15	15		Brachial arterial blood pressure; Aortic blood pressure; Arterial stiffness; carotid-femoral pulse wave velocity; 6-min walk test (6MWT); 10-m walk test (10MWT); and the Timed Up-and-Go (TUG) test; 30-sec chair-stand test; sit and reach test; functional reach test; grip strength	Combined aerobic and resistance exercise	Unsystematic physical activities, Korean Chess, to continue the normal daily activities	IG: 16 weeks, 3 days/week, 60' CG: no treatment	Usual care	Lee YH, Park SH, Yoon ES, Lee C-D, Wee SO, Fernhall B, et al. Effects of combined aerobic and resistance exercise on central arterial stiffness and gait velocity in patients with chronic poststroke hemiparesis. Am J Phys Med Rehabil 2015;94:687–95. https://doi.org/10.1097/PHM.0000000000000233.
2015	Mao	China	x	x		15	14		Brunel balance assessment; LE Fugl-Meyer; spatiotemporal gait parameters ; kineamtic and kinetic parameters during gait	Supported treadmill training + rehabilitation (standard care)	Conventional overground walking training + rehabilitation (standard care)	3 weeks (5 time per weekm; 120')	Conventional training	Mao Y-R, Lo WL, Lin Q, Li L, Xiao X, Raghavan P, et al. The Effect of Body Weight Support Treadmill Training on Gait Recovery, Proximal Lower Limb Motor Pattern, and Balance in Patients with Subacute Stroke. BioMed Res Int 2015;2015:175719. https://doi.org/10.1155/2015/175719.
2015	Ochi	Japan	x	x		13	13		Functional Ambulation Classification (FAC); peak torque of knee extensor; 10-m walking test (10MWT); Fugl-Meyer Assessment	Gait-assistance robot (GAR)-assisted gait training (OCGT) + Standard physical therapy	Overground conventional gait training (OCGT) + Standard physical therapy	Gait training: 4 weeks, 5 days/week, 20' Standard physical therapy: 4 weeks, 5 days/week, 60'	Standard physical therapy	Ochi M, Wada F, Saeki S, Hachisuka K. Gait training in subacute non-ambulatory stroke patients using a full weight-bearing gait-assistance robot: A prospective, randomized,

									(FMA); Functional Independence Measure (FIM)					open, blinded-endpoint trial. J Neurol Sci 2015;353:130–6. https://doi.org/10.1016/j.jns.2015.04.033.
2015	Oostra	Belgium		x	x	21	23		Movement Imagery Questionnaire-revised; Walking trajectory test; 10-m walk test (10MWT); Lower extremity Fugl-Meyer assessment scale (LE-FM)	Motor imagery training (MIT) + Standard rehabilitation programme	Muscle relaxation (MR) + Standard rehabilitation programme	MIT: 6 weeks, 5 days/week, 30' MR: 6 weeks, 5 days/week, 30' Standard rehabilitation programme: 6 weeks, 5 days/week, 180'	Standard rehabilitation programme	Oostra K, Oomen A, Vanderstraeten G, Vingerhoets G. Influence of motor imagery training on gait rehabilitation in sub-acute stroke: A randomized controlled trial. J Rehabil Med 2015;47:204–9. https://doi.org/10.2340/16501977-1908.
2015	Park	Republic of Korea		x	x	12	12		Muscle strenght of rectus femoris; tibilais anterior; gastrocnemius ; Timed Up and Go test ; step lenght	Step climbing exercise + basic exercise therapy	Stair gait exercises + basic exercise therapy	8 weeks (3 times per week; 15' + not specified amount of basic exercise therapy)	Basic exercise therapy	Park K-H, Kim D-Y, Kim T-H. The effect of step climbing exercise on balance and step length in chronic stroke patients. J Phys Ther Sci 2015;27:3515–8. https://doi.org/10.1589/jpts.27.3515.
2015	Sheffler	USA			x	54	56		Cadence; double support time; stride length; walking speed; peak hip flexion angle in swing; peak knee flexion angle in swing; peak hip extension during stance; dorsiflexion (DF) angle at initial contact; peak ankle DF angle in swing; peak ankle abduction angle in swing; peak ankle external rotation angle in swing peak anteriorposterior (AP) ground reaction force (GRF); peak hip power in preswing; peak ankle power at push-off(Quantitative	Peroneal nerve stimulator + usual care	Usual care	5 weeks, 2 days/week, 60' + 7 weeks, 3 days/week, 60'	Usual care	Sheffler LR, Taylor PN, Bailey SN, Gunzler DD, Buurke JH, IJzerman MJ, et al. Surface Peroneal Nerve Stimulation in Lower Limb Hemiparesis: Effect on Quantitative Gait Parameters. Am J Phys Med Rehabil 2015;94:341–57. https://doi.org/10.1097/PHM.0000000000000269.

								gait analysis); average daily time spent standing; average daily time spent stepping and average total steps per day						
2015	Shen	China	x	x		20	20		Lower extremity Fugl-Meyer assessment scale (LE-FM); 6-min walk test (6MWT); modified Barthel Index (mBI); left ventricular ejection fraction (LVEF)	Intensified walking training	Routine rehabilitation	5 weeks, 5-6 days/week	Routine rehabilitation	Shen D; Huang H; Yuan H; Ye S; Li M; Gu J; Wang Z. Therapeutic efficacy of intensified walk training under the electrocardiogram telemetry in stroke induced lower limb dysfunction patients with heart failure. Int J Clin Exp Med 2015 Sep 308916599-16605 2015.
2015	Wang	China	x	x		IG1: 18; IG2: 18; IG3: 18	18		Composite Spasticity Scale (CCS); Ankle Active Dorsiflexion Score; Timed Up and Go Test (TUG)	IG1: Full-movement neuromuscular electrical stimulation + conventional rehabilitation therapy IG2: Sensory threshold neuromuscular electrical stimulation + conventional rehabilitation therapy IG3: Motor threshold neuromuscular electrical stimulation + conventional rehabilitation therapy	Conventional rehabilitation therapy	IG: 4 weeks, 5 days/week, 2 session/days, 30' CG: N/R	Conventional rehabilitation therapy	Wang Y, Meng F, Zhang Y, Xu M, Yue S. Full-movement neuromuscular electrical stimulation improves plantar flexor spasticity and ankle active dorsiflexion in stroke patients: a randomized controlled study. Clin Rehabil 2016;30:577–86. https://doi.org/10.1177/0269215515597048.
2015	Zou	China			x	28	28		Blood glucose level; Serum lipids profiles; Body mass index.	Resistance training	conventional stroke physical therapy	8 weeks (3 sessions per week; 40')	Conventional stroke physical therapy	Zou J, Wang Z, Qu Q, Wang L. Resistance training improves hyperglycemia and dyslipidemia, highly prevalent among nonelderly, nondiabetic, chronically disabled stroke patients. Arch Phys Med Rehabil 2015;96:1291–6.

														https://doi.org/10.1016/j.apmr.2015.03.008.
2016	Chen	China	x			125	125		National Institutes of Health Stroke Scale (NIHSS); Fugl-Meyer Assessment (FMA); bedside swallowing assessment (BSA); videofluoroscopic swallowing study (VFSS); Mini-Mental State Examination (MMSE); Montreal Cognitive Assessment (MoCA); adverse reaction of acupuncture	Scalp acupuncture + conventional stroke rehabilitation care	Conventional stroke rehabilitation care	IG: 3 weeks, 6 days/week, 30' (Acupuncture) + 120' (conventional care) CG: 3 weeks, 6 days/week, 120' (conventional care)	Conventional stroke rehabilitation care	Chen L, Fang J, Ma R, Gu X, Chen L, Li J, et al. Additional effects of acupuncture on early comprehensive rehabilitation in patients with mild to moderate acute ischemic stroke: a multicenter randomized controlled trial. BMC Complement Altern Med 2016;16:226. https://doi.org/10.1186/s12906-016-1193-y.
2016	Du	China	x			IG1: 23; IG2: 23	23		Fugl-Meyer Assessment; Modified Rankin Scale; Medical Research Council score ; National Institute of Health Stroke Scale ; cortical excitability (Motor evoked potential)	IG1: 3-Hz ipsilesional rTMS + standard physical therapy IG2: 1-Hz contralesional rTMS + standard physcial therapy	Sham rTMS + standard physical therapy	IG and Sham: 5 days, once a day ; standard therapy: duration not specified, 60' each day	Standard physical therapy	Du J, Tian L, Liu W, Hu J, Xu G, Ma M, et al. Effects of repetitive transcranial magnetic stimulation on motor recovery and motor cortex excitability in patients with stroke: a randomized controlled trial. Eur J Neurol 2016;23:1666–72. https://doi.org/10.1111/ene.13105.
2016	Han	Republic of Korea	x			30	30		Brachial-ankle pulse wave velocity ; cardio-respiratory fitness (symptom-limited graded exercise stress test; A 12-lead electrocardiogram and expired-gas analysis) ; Korean Modified Barthel Index; Berg Balance Scale; Funciyonal Ambulatory Category ; Fugl-Meyer Assessment LL	Robot assisted gait therapy + conventional rehabilitation therapy	Conventional rehabilitation therapy	4 weeks (5 times per week ; IG: 30'+30'; CG: 60')	Conventional rehabilitation therapy	Han EY, Im SH, Kim BR, Seo MJ, Kim MO. Robot-assisted gait training improves brachial-ankle pulse wave velocity and peak aerobic capacity in subacute stroke patients with totally dependent ambulation: Randomized controlled trial. Medicine (Baltimore) 2016;95:e5078. https://doi.org/10.1097/MD.0000000000005078.
2016	In	Republic of Korea			x	15	15		Berg Balance Scale; e Functional Reaching Test; Timed Up and Go; static balance	Virtual reality reflection therapy plus conventional rehabilitation	Placebo virtual reality reflection therapy plus conventional rehabilitation	4 weeks (5 session per week; 30' + 30')	Conventional rehabilitation	In T, Lee K, Song C. Virtual Reality Reflection Therapy Improves Balance and Gait in Patients with Chronic

									ability; 10 m walking test speed.					Stroke: Randomized Controlled Trials. Med Sci Monit 2016;22:4046–53. https://doi.org/10.12659/MSM.898157.
2016	Salhab	Lebanon		x				18 (9/9)	Range of Motion (ankle); Fugl-Meyer Assessment for lower extremity; 10 Meter Walking test;	Mirror therapy with electrical stimulation	Conventional therapy	4 weeks plus 1 week of washout (4 times per week; 50')	Conventional therapy	Salhab G, Sarraj AR, Saleh S. Mirror therapy combined with functional electrical stimulation for rehabilitation of stroke survivors' ankle dorsiflexion. Annu Int Conf IEEE Eng Med Biol Soc IEEE Eng Med Biol Soc Annu Int Conf 2016;2016:4699–702. https://doi.org/10.1109/EMBC.2016.7591776.
2016	Sandberg	Sweden	x	x		29	27		Peak Work Rate with standard ergometer exercise stress test; 6MWT distance; Maximum Walking Speed 10 meters test; Timed Up and Go test; Single Leg Stance test; Euroqol-5D; Visual analog Scale on quality of life; Self-reported Stroke Impact Scale 2.0.	Aerobic exercise	General advice about physical training and activity at home	12 weeks IG: (2 sessions per week; 60'); CG no treatment	Standard care	Sandberg K, Kleist M, Falk L, Enthoven P. Effects of Twice-Weekly Intense Aerobic Exercise in Early Subacute Stroke: A Randomized Controlled Trial. Arch Phys Med Rehabil 2016;97:1244–53. https://doi.org/10.1016/j.apmr.2016.01.030.
2016	Zhang	China		x		18	18		Maximum isometric voluntary contraction strength paretic lower limb ; Cocontraction ratio ; Modified Ashworth Scale ; Functional Ambulation Category ; Barthel Index	Aquatic therapy	Standard physiotherapy	8 weeks (five time per week ; 40')	Conventional intervention Standard physiotherapy	Zhang Y, Wang Y-Z, Huang L-P, Bai B, Zhou S, Yin M-M, et al. Aquatic Therapy Improves Outcomes for Subacute Stroke Patients by Enhancing Muscular Strength of Paretic Lower Limbs Without Increasing Spasticity: A Randomized Controlled Trial. Am J Phys Med Rehabil 2016;95:840–9. https://doi.org/10.1097/PHM.0000000000000512.

2017	Kim	Republic of Korea			x	15	15		Korean version of Montreal Cognitive Assessment; Trail Making test; Stroop test; Handgrip strength (dynamometer); 10-Meter Walk test; Timed Up and Go test	Exercise protocol for handgrip + treadmill-based weight loading training + conventional therapy	Conventional physical therapy	6 weeks (; IG: 3 weeks for 15' + 3 times a day for 30'+ 5 times a week, 2 times a day for 60'; CG: 5 times a week, 2 times per day for 60')	Conventional physical therapy	Kim J, Yim J. Effects of an Exercise Protocol for Improving Handgrip Strength and Walking Speed on Cognitive Function in Patients with Chronic Stroke. Med Sci Monit 2017;23:5402–9. https://doi.org/10.12659/MSM.904723.
2017	An	Republic of Korea			x	13	13		Korean version of the Modified Barthel Index (KMBI); Modified Modified Ashworth scale; Isokinetic ankle strength; dorsiflexion passive range of motion; weight-bearing ability on the paretic limb during standing (Direction control score (%)) and gait (swing phase (%); single limb support phase (%); and double limb support phase (%))	Talocrural mobilization with movement	Conventional physiotherapy	IG: 5 weeks, 3 days/week, 30'+30' CG: 5 weeks, 3 days/week, 30'	Conventional physiotherapy	An C-M, Jo S-O. Effects of Talocrural Mobilization with Movement on Ankle Strength, Mobility, and Weight-Bearing Ability in Hemiplegic Patients with Chronic Stroke: A Randomized Controlled Trial. J Stroke Cerebrovasc Dis Off J Natl Stroke Assoc 2017;26:169–76. https://doi.org/10.1016/j.jstrokecerebrovasdis.2016.09.005.
2017	Arya	India		x	x	19	17		Brunnstrom recovery stages; Fugl-Meyer assessment; Lower extremity; Rivermead visual gait assessment; 10-meter walk test	Activity-based mirror therapy + conventional motor therapy	Conventional motor therapy	12 weeks (3-4 times per week; IG:30'+30', CG:60')	Conventional motor therapy	Arya KN, Pandian S, Kumar V. Effect of activity-based mirror therapy on lower limb motor-recovery and gait in stroke: A randomised controlled trial. Neuropsychol Rehabil 2019;29:1193–210. https://doi.org/10.1080/09602011.2017.1377087.
2017	Cha	Republic of Korea			x	11	11		Muscle activities of the lateral gastrocnemius and tibialis anterior via sEMG; Balance index (BI) scores (overall; anterior/posterior; medial/lateral); Timed Up and Go test (TUG)	Bad Ragaz Ring exercise + Conventional rehabilitation therapy	Conventional rehabilitation therapy	6 weeks, 3 days/week, 60'	Comprehensive rehabilitation therapy; Conventional rehabilitation therapy	Cha H-G, Shin Y-J, Kim M-K. Effects of the Bad Ragaz Ring Method on muscle activation of the lower limbs and balance ability in chronic stroke: A randomised controlled trial. Hong Kong Physiother J 2017;37:39–45.

														https://doi.org/10.1016/j.hkpj.2017.02.001.
2017	Choi	Republic of Korea			x	IG1: 12; IG2: 12	12		COP displacemnt ; weight bearing symmetry ; balance ability (Functional Reach Test; modified Functional Reach Test; Timed Up and Go test)	IG1: Game-based constraint-induced movement therapy + traditional physical therapy IG2: General game-based training + traditional physical therapy	Traditional physical therapy	4 weeks (IG1-2: 3 times per week for 30' + 5 times per week of traditional physical theraoy for 60' ; CG: 5 times per week of traditional physical therpy for 60')	Traditional physical therapy	Choi H-S, Shin W-S, Bang D-H, Choi S-J. Effects of Game-Based Constraint-Induced Movement Therapy on Balance in Patients with Stroke: A Single-Blind Randomized Controlled Trial. Am J Phys Med Rehabil 2017;96:184–90. https://doi.org/10.1097/PHM.0000000000000567.
2017	De	India		x	x	IG1: 15; IG2: 15	N/A		Modified Ashworth Scale; 10 Meter Walk test ; Fugl-Meyer assessment LE	IG1: mirror therapy + conventional therapy IG2: mental imagery + conventional therapy	N/A	4 weeks (5 days per week; 60')	Conventional therapy	De S, Chopra C, Mehta DD, Mehndiratta MM. Comparison between Mirror Therapy and Mental Imagery in Improving Ankle Motor Recovery in Sub Acute Stroke Patients. Indian J Physiother Occup Ther - Int J 2017;11:169. https://doi.org/10.5958/0973-5674.2017.00097.1.
2017	de Lima Gomes	Brazil			x	7	6		Modified Ashworth Scale; Fugl-Meyer Assessment; Time Up and Go test; Postural Stroke Scale; Barthel Index; Confidence and Balance Scale; Body Mass Measurement.	Gait training on the treadmill	conventional treatment	6 weeks (2 sessions per week; 20')	Conventional treatment	de Lima Gomes W, de Nadai Dias LI, Guimarães RP, Stival CM, Faria G da R, Bovi ACN, et al. Effects of Treadmill Training in chronic hemiparetic: a randomized, double-blind clinical trial. Man Ther Posturology Rehabil J 2017;15:1–5. https://doi.org/10.17784/mtprehabjournal.2017.15.530.
2017	Dujović	Serbia	N/A	N/A	N/A	8	8		Spedd of 10MWT; Fugl-Meyer Assessment; Berg balance scale; Barthel index.	conventional stroke rehabilitation plus functional electrical stimulation of plantarflexor and dorsiflexor	conventional stroke rehabilitation	4 weeks (5 days per week; 60')	Conventional stroke rehabilitation Conventional rehabilitation program	Dujović SD, Malešević J, Malešević N, Vidaković AS, Bijelić G, Keller T, et al. Novel multi-pad functional electrical stimulation in stroke patients: A single-blind randomized study. NeuroRehabilitation

														2017;41:791–800. https://doi.org/10.3233/NRE-172153.
2017	In	Republic of Korea			x	15	15		Fugl-Meyer Assessment; Timed Up and Go test; 10 meter walk test; Performance-oriented mobility assessment (or Tinetti Mobility Test)	Treadmill training with Thera-Band + conventional exercise program	Treadmill training + conventional exercise program	4 weeks (fiver times per week; 30'+30')	Conventional physical therapy Conventional exercise program	In T, Jin Y, Jung K, Cho H. Treadmill training with Thera-Band improves motor function, gait and balance in stroke patients. NeuroRehabilitation 2017;40:109–14. https://doi.org/10.3233/NRE-161395.
2017	Ivey	USA			x	14	16		Endurance (maximum number of leg press repetitions possible at 70% of one repetition maximum) and strenght (one repetition maximum) for both paretic and non-paretic legs; 6 minutes walking test; Peak Oxygen Consumption; 10 minutes walking test	Strength training of lower extermities	Supervised stretching exercises (component of usual care)	12 weeks (3 times per week, 20 repetition set)	Usual care Standard physical therapy	Ivey FM, Prior SJ, Hafer-Macko CE, Katzel LI, Macko RF, Ryan AS. Strength Training for Skeletal Muscle Endurance after Stroke. J Stroke Cerebrovasc Dis 2017;26:787–94. https://doi.org/10.1016/j.jstrokecerebrovasdis.2016.10.018.
2017	Jung	Republic of Korea		x	x	20	21		Balance ability (Nintendo Wii platform); isometric strength in the extensor of hip; knee and ankle (dynamometer); Composite Spasticity Score	TENS + sit-to-stand training + conventional therapy	ShamTENS + sit-to-stand training + conventional therapy	6 weeks (5 times a week; 30'+ 15' + 60')	Conventional therapy	Jung K-S, In T-S, Cho H. Effects of sit-to-stand training combined with transcutaneous electrical stimulation on spasticity, muscle strength and balance ability in patients with stroke: A randomized controlled study. Gait Posture 2017;54:183–7. https://doi.org/10.1016/j.gaitpost.2017.03.007.
2017	Lee	Republic of Korea	N/A	N/A	N/A	IG1: 12; IG2: 11; IG3: 12			balance index through the Biodex Balance System; Modified Functional Ambulation Profile;	IG1: Action observation therapy with activity plus general physical therapy; IG 2: Mirror therapy with activity plus general physical therapy; IG 3: Action observation therapy		6 weeks (3 sessions per week; 30' + 2 sessions per week; 30')	General physical therapy	Lee HJ, Kim YM, Lee DK. The effects of action observation training and mirror therapy on gait and balance in stroke patients. J Phys Ther Sci 2017;29:523–6. https://doi.org/10.1589/jpts.29.523.

										plus general physical therapy.				
2017	Nikamp	Netherlands	x	x		8	12		Spatio-temporal parameters of gait; Movement and angles of lower limb joints during gait; 10-metre walk test; Berg Balance Scale; 6-minute walk test; Timed Up and Go Test; Stairs Test; Functional Ambulation Catergory; Rivermead Mobility Index; Barthel Index	Early AFO-provision + usual care (Dutch guidelines)	Delayed AFO-provision + usual care (Dutch guidelines)	11 weeks	Usual care (Dutch guidelines)	Nikamp CDM, Hobbelink MSH, van der Palen J, Hermens HJ, Rietman JS, Buurke JH. A randomized controlled trial on providing ankle-foot orthoses in patients with (sub-)acute stroke: Short-term kinematic and spatiotemporal effects and effects of timing. Gait Posture 2017;55:15–22. https://doi.org/10.1016/j.gaitpost.2017.03.028.
2017	Rasheeda	India	x			38	36		30-second sit to stand test; weight bearing through the paretic lower limb; Brunnstrom stage of motor control in lower limb	Conventional physiotherapy with Swiss Ball	Conventional physiotherapy	10 days for 40'	Conventional physiotherapy	Vadnagarwala R, Ramachandran S. The Effect of Swiss Ball Therapy on Sit-to-Stand Function, Paretic Limb Weight Bearing and Lower Limb Motor Score in Patients with Hemiplegia. Int J Physiother 2017;4. https://doi.org/10.15621/ijphy/2017/v4i6/163917.
2017	Wang	China	x	x		18	18		Brunnstrom for lower limb; Berg Balance Scale; Functional Ambulation Category; Funtional Indipendence Measure walking part.	Assisted mirror therapy with visual feedback or motor imagery plus conventional rehabilitation treatment	Mirror therapy without visaul feedback and motor imagery plus conventional rehabilitation tratment	6 weeks (5 days per week; 40' + 120'/180')	Routine rehabilitation treatment Conventional rehabilitation treatment	Wang H, Zhao Z, Jiang P, Li X, Lin Q, Wu Q. Effect and mechanism of mirror therapy on rehabilitation of lower limb motor function in patients with stroke hemiplegia. Biomed Res India 2017;28:10165–70.
2017	Xu Q	China		x		IG1: 23; IG2: 23	23		10 meter walk test; Brunnstrom stages of lower extremity; Modified Ashworth Scale; Range of Motion (ankle joint)	Mirror therapy + neuromuscular electricla stimulation + conventional stroke rehabilitation Mirror therapy + conventional stroke rehabilitation	Conventional stroke rehabilitation	4 weeks (5day per week for 270')	Conventional stroke rehabilitation	Xu Q, Guo F, Salem HMA, Chen H, Huang X. Effects of mirror therapy combined with neuromuscular electrical stimulation on motor recovery of lower limbs and walking ability of patients with stroke: a randomized controlled study. Clin Rehabil

														2017;31:1583–91. https://doi.org/10.1177/0269215517705689.
2018	Dalal	India	N/A	N/A	N/A	16	16		Knee hyperextention and ankle dosriflexion	Prowling along with proprioceptice training + routine physiotherapy	Routine physiotherapy	6 session (IG: 15-20' + 45'-60' ; CG: rest + 45-60')	Routine physiotherapy	Dalal KK, Joshua AM, Nayak A, Mithra P, Misri Z, Unnikrishnan B. Effectiveness of prowling with proprioceptive training on knee hyperextension among stroke subjects using videographic observation- a randomised controlled trial. Gait Posture 2018;61:232–7. https://doi.org/10.1016/j.gaitpost.2018.01.018.
2018	Du	China	x			IG1: 20; IG2: 20	20		Upper extremityFugl–Meyer Assessment; MedicalResearch Council scale; National Institutes of Health Stroke Scale; Modified Rankin Scale; motor evoked potential; fMRI.	IG 1: High Frequency-repetitive trans-cranial magnetic stimulation plus standardize physiotherapy; IG 2: Low Frequency-repetitive trans-cranial magnetic stimulation plus standardize physiotherapy.	Sham repetitive trans-cranial magnetic stimulation plus standardize physiotherapy	1 week (5 sessions; time not specfied + 5 sessions; 60')	Standardized physiotherapy protocol	Du J, Yang F, Hu J, Hu J, Xu Q, Cong N, et al. Effects of high- and low-frequency repetitive transcranial magnetic stimulation on motor recovery in early stroke patients: Evidence from a randomized controlled trial with clinical, neurophysiological and functional imaging assessments. NeuroImage Clin 2019;21:101620. https://doi.org/10.1016/j.nicl.2018.101620.
2018	Dubey	India		x	x	17	17		Trunk Impairment Scale 2.0; Fugl Meyer Assessment-Lower Extremity; Hand-held dynamometer; 10-m walk test; Modified Barthel Index	Pelvic stability training	Standard physiotherapy training	6 weeks (3 sessions per week; 60' per session)	Standard physiotherapy	Dubey L, Karthikbabu S, Mohan D. Effects of Pelvic Stability Training on Movement Control, Hip Muscles Strength, Walking Speed and Daily Activities after Stroke: A Randomized Controlled Trial. Ann Neurosci 2018;25:80–9. https://doi.org/10.1159/000486273.
2018	Elhakk	Egypt			x	15	15		Berg balance test; functional independence	Traditional physical therapy plus task oriented training and	Traditional physical therapy plus task oriented training	not specified (sessions not specified; IG 30'; CG: 45')	Traditional physical therapy	Elhakk SMA, Ragab WM, Zakaria HM, Faggal MS, Taha SI, Badawy WM, et

									measure; Functional Reach Test; Timed up-and-Go; Evaluation of center of pressure	Wii balance board therapy				al. Task oriented approach via virtual reality for improving postural control in stroke patients. Biosci Res 2018;15:3926–33.
2018	Erbil	Turkey			x	32	16		Modified Ashworth Scale (MAS); Tardieu Scale; Berg Balance Scale (BBS); Timed Up and Go test (TUG); and Rivermead Visual Gait Assessment	Robot-assisted training (RAT) + physical therapy + BoNT-A injections	Physical therapy + BoNT-A injections	3 weeks, 5 days/week, 90'	Conventional physical therapy	Erbil D, Tugba G, Murat TH, Melike A, Merve A, Cagla K, et al. Effects of robot-assisted gait training in chronic stroke patients treated by botulinum toxin-a: A pivotal study. Physiother Res Int 2018;23:e1718. https://doi.org/10.1002/pri.1718.
2018	Ferreira LAB	Brazil			x	12	12		Kinematic and spatiotemporal gait parameters	Insole with corrective elements + conventional physical therapy	Placebo insole + conventional physical therapy	12 weeks	Conventional therapy	Ferreira LAB, Cimolin V, Neto HP, Grecco LAC, Lazzari RD, Dumont AJL, et al. Effect of postural insoles on gait pattern in individuals with hemiparesis: A randomized controlled clinical trial. J Bodyw Mov Ther 2018;22:792–7. https://doi.org/10.1016/j.jbmt.2017.08.004.
2018	Ganesh	India		x	x	IG1: 27; IG2: 30	26		Active and passive ankle ROM; Ankle plantar flexor spasticity modified Ashworth scale; Modified Emory Functional Ambulation Profile.	IG1: Faradic current and general and individually directed task-oriented approach training plus standard multidisciplinary rehabilitation programme; IG 2: Russian currents and general and individually directed task-oriented approach training plus standard multidisciplinary rehabilitation programme.	General and individually directed task-oriented approach training plus standard multidisciplinary rehabilitation programme	6 weeks IG (5 sessions per week; 10' + 80') CG (5 sessions per week; 80')	Standard multidisciplinary rehabilitation programme	Ganesh GS, Kumari R, Pattnaik M, Mohanty P, Mishra C, Kaur P, et al. Effectiveness of Faradic and Russian currents on plantar flexor muscle spasticity, ankle motor recovery, and functional gait in stroke patients. Physiother Res Int 2018;23:e1705. https://doi.org/10.1002/pri.1705.
2018	Ghasemi (a)	Iran		x	x	15	15		Modified modified ashworth scale;	Functional stretching exercises	Routine physical therapy	4 weeks (3 times a week)	Routine physical therapy	Ghasemi E, Khademi-Kalantari K, Khalkhali-

									achilles deep tendon reflex; ankle range of motion; ankle clonus; 10-meter walking timed test; Timed Up and Go test; visual analogue scale (experience regarding the impact of ankle spasticity on ambulation)					Zavieh M, Rezasoltani A, Ghasemi M, Baghban AA, et al. The effect of functional stretching exercises on functional outcomes in spastic stroke patients: A randomized controlled clinical trial. J Bodyw Mov Ther 2018;22:1004–12. https://doi.org/10.1016/j.jbmt.2017.09.021.
2018	Ghasemi (b)	Iran		x	x	30	15		Modified Modified Ashworth scale; Dorsiflexion passive range of movement; H-reflex; M-wave; B-mode.	Functional stretching training	routine physical therapy program	IG 4 weeks (3 sessions per week); CG not specified	Routine physical therapy program	Ghasemi E, Khademi-Kalantari K, Khalkhali-Zavieh M, Rezasoltani A, Ghasemi M, Akbarzadeh Baghban A, et al. The Effect of Functional Stretching Exercises on Neural and Mechanical Properties of the Spastic Medial Gastrocnemius Muscle in Patients with Chronic Stroke: A Randomized Controlled Trial. J Stroke Cerebrovasc Dis Off J Natl Stroke Assoc 2018;27:1733–42. https://doi.org/10.1016/j.jstrokecerebrovasdis.2018.01.024.
2018	Huang	Taiwan	x	x		18	20		Timed Up and Go test; Postural Assessment Scale fo Stroke; Fugl-Meyer Assessment (LE); Barthel Index; Chinese version of Stroke Specific Quality of Life Scale	1-Hz repetitive transcranial magnetic stimulation + regular physioptherapy	Sham stimulation + regular physioptherapy	3 weeks (15 daily session ; 15'+ 45')	Regular physiotherapy	Huang Y-Z, Lin L-F, Chang K-H, Hu C-J, Liou T-H, Lin Y-N. Priming With 1-Hz Repetitive Transcranial Magnetic Stimulation Over Contralesional Leg Motor Cortex Does Not Increase the Rate of Regaining Ambulation Within 3 Months of Stroke: A Randomized Controlled Trial. Am J Phys Med Rehabil 2018;97:339–45. https://doi.org/10.1097/PHM.0000000000000850.

2018	Kim	Republic of Korea			x	IG1: 10; IG2: 10	10		Muscle strenght in the knee externsor and flexor of paretic and non-paretic side	IG1: mirror lower extremity muscle strenght exercise IG2:mirror lower extremity exercise	Non-mirror lower extremity exercise	4 weeks (5 day per week, 5 sets 30 times a day)	General physical therapy	Kim M-K, Choe Y-W, Shin Y-J, Peng C, Choi E-H. Effect of mirror use on lower extremity muscle strength of patients with chronic stroke. J Phys Ther Sci 2018;30:213–5. https://doi.org/10.1589/jpts.30.213.
2018	Klomjai	Thailand		x				19 (10/9)	Knee extensor strenght; Timed Up and Go test ; Five times sit to stand test	Real transcranial direct current stimulation + conventional physical therapy	Sham transcranial direct current stimulation + conventional physical therapy	2 session (IG 20' + 60' ; CG: 2'+ 60')	Conventional physical therapy	Klomjai W, Aneksan B, Pheungphrarattanatrai A, Chantanachai T, Choowong N, Bunleukhet S, et al. Effect of single-session dual-tDCS before physical therapy on lower-limb performance in sub-acute stroke patients: A randomized sham-controlled crossover study. Ann Phys Rehabil Med 2018;61:286–91. https://doi.org/10.1016/j.rehab.2018.04.005.
2018	Lee	Republic of Korea			x	23	22		Gait analysis; Timed Up and Go test; Berg Balance Scale; Fugl–Meyer Assessment	Gait training with bilateral rhythmic auditory stimulation plus conventional rehabilitation	Gait training without bilateral rhythmic auditory stimulation plus conventional rehabilitation	6 weeks (5 days per week, 30' + 60' per session)	Conventional rehabilitation	Lee S, Lee K, Song C. Gait Training with Bilateral Rhythmic Auditory Stimulation in Stroke Patients: A Randomized Controlled Trial. Brain Sci 2018;8:164. https://doi.org/10.3390/brainsci8090164.
2018	Maciaszek	Poland		x		10	10		one-leg standing test; timed up & go test	Maintenance of body balance	Standard therapeutic program	2 weeks	Standard therapeutic program	Maciaszek J. Effects of Posturographic Platform Biofeedback Training on the Static and Dynamic Balance of Older Stroke Patients. J Stroke Cerebrovasc Dis 2018;27:1969–74. https://doi.org/10.1016/j.jstrokecerebrovasdis.2018.02.055.
2018	Mainka	Germany	x	x		IG1: 11; IG2: 13	11		Gait velocity; cadence; stride length; gait symmetry; endurance; postural	IG1: Rhythmic-auditory stimulation during treadmill training plus conventional	Neurodevelopmental treatment plus conventional physiotherapy	3 weeks IG (sessions not specified; 15'/20' + 30'/60'); CG (sessions not specified; 30' + 30'/60')	Conventional physiotherapy	Mainka S, Wissel J, Völler H, Evers S. The Use of Rhythmic Auditory Stimulation to Optimize Treadmill Training for

									stability; fast gait speed test; 3-minute walking test; instrumental evaluation of balance	physiotherapy; IG2: Treadmill training plus conventional physiotherapy				Stroke Patients: A Randomized Controlled Trial. Front Neurol 2018;9:755. https://doi.org/10.3389/fneur.2018.00755.
2018	Nikamp	Netherlands	x	x		16	17		Pelvic obliquity; hip ab/adduction; knee flexion/extension; ankle kinematics; walking speed;	Early AFO-provision + usual care (Dutch guidelines)	Delayed AFO-provision + usual care (Dutch guidelines)	26 weeks	Usual care (Dutch guidelines)	Nikamp CDM, van der Palen J, Hermens HJ, Rietman JS, Buurke JH. The influence of early or delayed provision of ankle-foot orthoses on pelvis, hip and knee kinematics in patients with sub-acute stroke: A randomized controlled trial. Gait Posture 2018;63:260–7. https://doi.org/10.1016/j.gaitpost.2018.05.012.
2018	Pan	China	N/A	N/A	N/A	45	41		Fugl–Meyer assessment; NDS; Barthel Index.	Routine drug therapy plus rehabilitation therapy on the basis of routine therapy	Routine drug therapy plus nothing else specified	IG 50 days (1/2 sessions per day; 30'/60'); CG not specified	Routine therapy	Pan X-L. Efficacy of early rehabilitation therapy on movement ability of hemiplegic lower extremity in patients with acute cerebrovascular accident. Medicine (Baltimore) 2018;97:e9544. https://doi.org/10.1097/MD.0000000000009544.
2018	Park	Republic of Korea		x	x	10	10		Ankle dorsiflexion passive range of motion; static balance ability; Berg balance scale; gait parameters (speed; cadence; support periods; step length)	Mobilization with movement + standard physical therapy	Static muscle stretching + standard physical therapy	4 weeks (3 times per week for about 8'+ 5 times per week for 30')	Standard rehabilitation therapy	Park D, Lee J-H, Kang T-W, Cynn H-S. Four-week training involving ankle mobilization with movement versus static muscle stretching in patients with chronic stroke: a randomized controlled trial. Top Stroke Rehabil 2019;26:81–6. https://doi.org/10.1080/10749357.2018.1550614.
2018	Son	Republic of Korea			x	10	10		Muscle activity; Timed Up and Go test; 10-meter walk test	Self-observation training + general reheabilitation training	General reheabilitation training	4 weeks (IG: 3 day a week for 30' + 5 day a week for 30' ; CG: 5 day a week for 30')	General rehabilitation training Routine exercise therapy	Son Y-L, Kim J-W. The effects of mirror neuron system-based self-observation training on lower limb muscle activity and dynamic

														balance in patients with chronic stroke. J Phys Ther Sci 2018;30:1241–4. https://doi.org/10.1589/jpts.30.1241.
2018	Tsaih	Taiwan		x	x	IG 1: 11 IG 2: 13	9		Tibialis Anterior strength; Limit of Stability through Smart Balance Master System; Timed Up and Go test; six-minute walk test.	IG 1: Variable practice of Tibialis Anterior Electromyographic biofeedback plus General Physical Therapy; IG 2: Constant practice of Tibialis Anterior Electromyographic biofeedback plus General Physical Therapy.	Upper extremity exercise program plus General Physical Therapy	6 weeks (18 sessions per 6 weeks; 40' + 2/3 sessions per week, time not specified)	Routine outpatient physical therapy General Physical Therapy	Tsaih P-L, Chiu M-J, Luh J-J, Yang Y-R, Lin J-J, Hu M-H. Practice Variability Combined with Task-Oriented Electromyographic Biofeedback Enhances Strength and Balance in People with Chronic Stroke. Behav Neurol 2018;2018:1–9. https://doi.org/10.1155/2018/7080218.
2018	Zhang	China	x	x	x	72	71		Clinical Nerve Function Limitation Score; Barthel Index; Quality of Life Scale; Manual Muscle Testing.	high-quality nursing services in addition to the conventional rehabilitation therapy	conventional rehabilitation therapy	12 weeks (not specified)	Conventional rehabilitation	Zhang Y, Qin L, Shi Y, Zhong C. Effects of high-quality nursing services on the neurological functions and abilities of daily living of stroke patients. NeuroQuantology 2018;16:7–12. https://doi.org/10.14704/nq.2018.16.5.1327.
2018	Zhao	China	x	x		14	14		Fugl-Meyer Assessment; Modified Barthel Index	Scalp acupuncture + low frequency rTMS + routine rehabilitation treatment (rrt)	Scalp acupuncture + routine rehabiltation treatment (rrt)	2 weeks (IG: 14 daily session for 20'+ rrt for not specified amount of time; CG: 14 daily session for 40' + rrt for not specified amount of time	Routine rehabilitation treatment Regular rehabilitation treatment	Zhao N, Zhang J, Qiu M, Wang C, Xiang Y, Wang H, et al. Scalp acupuncture plus low-frequency rTMS promotes repair of brain white matter tracts in stroke patients: A DTI study. J Integr Neurosci 2018;17:125–39. https://doi.org/10.3233/JIN-170043.
2018	Zheng	China	x			IG1: 20; IG2: 20	20		Lower extremity Fugl-Meyer Movement Assessment; Postural Assessment Scale for Stroke Patients; Berg Balance Scale; Brunel Balance Assessment; modified Barthel	IG 1: Rehabilitation standard program plus 4 channels Functional Electrical Stimulation; IG 2: Rehabilitation standard program plus 2 channels	Rehabilitation standard program plus Placebo Functional Electrical Stimulation	3 weeks (5 sessions per week; 120' + not specified)	Rehabilitation standard programs	Zheng X, Chen D, Yan T, Jin D, Zhuang Z, Tan Z, et al. A Randomized Clinical Trial of a Functional Electrical Stimulation Mimic to Gait Promotes Motor Recovery and Brain Remodeling in Acute Stroke. Behav

									Index; Functional magnetic resonance images.	Functional Electrical Stimulation.				Neurol 2018;2018:1–10. https://doi.org/10.1155/2018/8923520.
2019	Ancona	Italy			x	22	22		National Institutes of Health Stroke Scale; Tinetti scale; Functional Independence Measure.	Conventional neurorehabilitation plus verticalization with Erigo®	Conventional neurorehabilitation	4 weeks (5 sessions per week; 75')	Conventional neurorehabilitation	Ancona E, Quarenghi A, Simonini M, Saggini R, Mazzoleni S, De Tanti A, et al. Effect of verticalization with Erigo® in the acute rehabilitation of severe acquired brain injury. Neurol Sci Off J Ital Neurol Soc Ital Soc Clin Neurophysiol 2019;40:2073–80. https://doi.org/10.1007/s10072-019-03917-0.
2019	Chung	Republic of Korea		x		19	19		Digit span test; Korean version of Mini-Mental State Examination; Global Deterioration Scale; Berg Balance Scale; Time Up and Go Test; 10 meters Walking Test; 6 minutes Walking Test; Modified Barthel Index	Task-specific lower extremity training	Conventional physical therapy	3 weeks (5 sessions per week; 60' per session)	Conventional physical therapy	Chung SH, Kim JH, Yong SY, Lee YH, Park JM, Kim SH, et al. Effect of Task-Specific Lower Extremity Training on Cognitive and Gait Function in Stroke Patients: A Prospective Randomized Controlled Trial. Ann Rehabil Med 2019;43:1–10. https://doi.org/10.5535/arm.2019.43.1.1.
2019	de Sousa	Australia	x	x		15	15		Clinicians'impressions of sit-to-standchange; Sit-to-stand ability using thesit-to-stand item of the Mobility Scale for Acute Stroke Patients; Strenght of lower limb muscles; grosslower limb extension strength; the Goal Attainment Scale; andranking of change in ability to move from sitting to standing.	sit-to-stand training was based on the principles oftask-specific motor training plus usual care	usual care	2 weeks IG: (6 days per week; 60' + 2 sessions per weekday; 60'); CG (2 sessions per weekday; 60')	Usual care	de Sousa DG, Harvey LA, Dorsch S, Varettas B, Jamieson S, Murphy A, et al. Two weeks of intensive sit-to-stand training in addition to usual care improves sit-to-stand ability in people who are unable to stand up independently after stroke: a randomised trial. J Physiother 2019;65:152–8. https://doi.org/10.1016/j.jphys.2019.05.007.
2019	dos Santos	Brazil		x	x	10	10		Motor cortex excitability (intensity of the magnetic stimulator output; expressed as	Repetitive Transcranial Magnetic Stimulation (rTMS) + physiotherapy	Sham Repetitive Transcranial Magnetic Stimulation + physiotherapy	10 session (rTMS or sham-rTMS application + 30')	Physiotherapy based on Clinical practice guidelines for stroke patients	dos Santos RBC, Galvão SCB, Frederico LMP, Amaral NSL, Carneiro MIS, de Moura Filho AG, et al. Cortical and spinal

								percentage of maximal stimulator output inducing motor evoked potential amplitude of 1 mV elicited by single-pulse TMS); excitability of spinal cord (H-reflex amplitude); Modified Ashworth scale					excitability changes after repetitive transcranial magnetic stimulation combined to physiotherapy in stroke spastic patients. Neurol Sci 2019;40:1199–207. https://doi.org/10.1007/s10072-019-03765-y.
2019	Gamez	Spain	x			20	20		Upper and lower limb EMG signal; Barthel index; Fugl-Meyer Assessment; Daniels and Wothingham muscle test; Kendal Manual Muscle test;Lovett test	Surface electromyography-biofeedback + conventional physiotherapy	Conventional physiotherapy	12 weeks (24 session, 2 times per week; 60')	Conventional physiotherapy	Gámez AB, Hernandez Morante JJ, Martínez Gil JL, Esparza F, Martínez CM. The effect of surface electromyography biofeedback on the activity of extensor and dorsiflexor muscles in elderly adults: a randomized trial. Sci Rep 2019;9:13153. https://doi.org/10.1038/s41598-019-49720-x.
2019	Guan	China	N/A	N/A	N/A	63	65		Fugl-Meyer Assessment; National Institute of Health stroke scale; Family APGAR index	Routine care plus continuation of care	Routine care plus no treatment	12 weeks (not specified)	Routine care Routine neurological care	Guan Y, Guo N, Gao H, Sun J, Sun L, Liu X, et al. Study on application of continuous nursing in rehabilitation period of stroke patients. Acta Medica Mediterr 2019;35:539–43. https://doi.org/10.19193/0393-6384_2019_1s_83.
2019	Handelzalts	Israel	x	x		18	16		Multiple step-threshold and fall threshold; Berg Balance Scale; 6-minute walk test; 10-meter walk test; Activity-specific Balance Confidence	Perturbation-based balance training + regular rehabilitation	weight shifting and gait training + regular rehabilitation	2.5 weeks (12 session for 30' + not specifeid amount of time of regular rheabilitaion)	Regular rehabilitation	Handelzalts S, Kenner-Furman M, Gray G, Soroker N, Shani G, Melzer I. Effects of Perturbation-Based Balance Training in Subacute Persons With Stroke: A Randomized Controlled Trial. Neurorehabil Neural Repair 2019;33:213–24. https://doi.org/10.1177/1545968319829453.
2019	Hsieh	Taiwan		x	x	28	28		10-Metre Test of Walking; Centre of	Video Games rehabilitation plus	Regular physiotherapy	10 weeks IG (1 session per week; 30' + not	Regular physiotherapy	Hsieh H-C. Training by Using an Adaptive Foot

									Pressure sway kinematics.	regular physiotherapy		specified) CG (1 session per week; not specified)		Switch and Video Games to Improve Balance and Mobility Following Stroke: A Randomised Controlled Trial. Brain Impair 2019;20:16–23. https://doi.org/10.1017/BrImp.2018.15.
2019	Kim HY	Republic of Korea		x	x			19 (10/9)	Berg Balance Scale; Changes in center-of-pressure; Trunk impairment scale; Fugl-Meyer Assessment; Functional Ambulation Category; 10-m walk test.	robot-assisted gait training plus conventional physical therapy	conventional physical therapy	4 weeks (5 sessions per week; 60' per session)	Conventional physical therapy	Kim HY, Shin J-H, Yang SP, Shin MA, Lee SH. Robot-assisted gait training for balance and lower extremity function in patients with infratentorial stroke: a single-blinded randomized controlled trial. J NeuroEngineering Rehabil 2019;16:99. https://doi.org/10.1186/s12984-019-0553-5.
2019	Kim JY	Republic of Korea	x	x		28	30		Functional Ambulation Category; Motricity Index-Lower; 10 Meter Walk Test; Modified Barthel Index; Rivermead Mobility Index; Berg Balance Scale	Morning Walk (lower limb rehabilitation robot) + conventional physiotherapy	Conventional physiotherapy	3 weeks (15 session, 5 time per week; IG: 30' + 60' , CG: 90')	Conventional physiotherapy	Kim J, Kim DY, Chun MH, Kim SW, Jeon HR, Hwang CH, et al. Effects of robot-(Morning Walk®) assisted gait training for patients after stroke: a randomized controlled trial. Clin Rehabil 2019;33:516–23. https://doi.org/10.1177/0269215518806563.
2019	Lee	Republic of Korea			x	21	21		Fugl–Meyer Assessment LE; static sitting balance (posturpgraphy); modified functional reach test (dynamic sitting balance); velocity; cadence; stride time; and step time; tride length and step length (gait ability)	Speed-interactive pedaling training + conventional rehabilitation program	Cycle training + conventional rehabilitation program	6 weeks (5 days per week; 40' + 60')	Conventional therapy Conventional rehabilitation program	Lee K. Speed-Interactive Pedaling Training Using Smartphone Virtual Reality Application for Stroke Patients: Single-Blinded, Randomized Clinical Trial. Brain Sci 2019;9:295. https://doi.org/10.3390/brainsci9110295.
2019	Nam	Republic of Korea	x	x	x	20	20		functional ambulation category; Rivermead mobility index; 10-meter walk test; 6-minute walk test; motricity index;	electromechanical-assisted gait training plus physical therapy	physical therapist-assisted gait training plus physical therapy	4 weeks (5 days per week; 60')	Traditional physical therapy	Nam Y-G, Lee JW, Park JW, Lee HJ, Nam KY, Park JH, et al. Effects of Electromechanical Exoskeleton-Assisted Gait Training on Walking

									Berg balance scale. and modified Barthel index					Ability of Stroke Patients: A Randomized Controlled Trial. Arch Phys Med Rehabil 2019;100:26–31. https://doi.org/10.1016/j.apmr.2018.06.020.
2019	Nordin	Malaysia			x	45	46		Health-related quality of life; Rivermead Mobility Index; Berg Balance Scale; Five Times Sit to Stand; Timed 10 meter walk test.	carer-assisted therapy conducted at home	usual therapist-delivered therapy implemented in out-patient hospital setting	12 weeks IG (3 sessions per week; 45'/60'); CG (1 session per week; 120')	Usual therapy	Nordin NAM, Aziz NA, Sulong S, Aljunid SM. Effectiveness of home-based carer-assisted in comparison to hospital-based therapist-delivered therapy for people with stroke: A randomised controlled trial. NeuroRehabilitation 2019;45:87–97. https://doi.org/10.3233/NRE-192758.
2019	Park	Republic of Korea			x	19	19		Ankle Dorsi-Flexion Passive ROM; Spatio-temporal gait parameters; Fall risk.	Ankle Self-mobilization with movement plus Conventional physiotherapy	Calf muscle stretching training plus Conventional physiotherapy	4 weeks (3 sessions per week; 15' + 30')	Conventional physiotherapy	Park D, Cynn H-S, Yi C, Choi WJ, Shim J-H, Oh D-W. Four-week training involving self-ankle mobilization with movement versus calf muscle stretching in patients with chronic stroke: a randomized controlled study. Top Stroke Rehabil 2020;27:296–304. https://doi.org/10.1080/10749357.2019.1690831.
2019	Park	Republic of Korea		x		13	12		Lower limb Fugl–Meyer Assessment; Timed Up & Go test; 10 m walk test; Modified barthel index.	motor imagery training and electromyography-triggered electrical stimulation plus conventional rehabilitation therapy	motor imagery training plus conventional rehabilitation therapy	not specified	Conventional rehabilitation therapy	Park J-S, Choi J-B, Lee G, Lee S-H, Jung Y. EFFECT OF MOTOR IMAGERY TRAINING IN COMBINATION WITH ELECTROMYOGRAPHY-TRIGGERED ELECTRICAL STIMULATION IN STROKE WITH HEMIPLEGIA PATIENTS: A RANDOMIZED CONTROLLED STUDY. J Mech Med Biol 2019;19:1940062. https://doi.org/10.1142/S0219519419400621.

2019	Pradines	France			x	12	11		fascicle length in the soleus muscle measured by ultrasound; other ultrasonographic architectural parameters: soleus thickness; gastrocnemius fascicle length; gastrocnemius thickness; maximal extensibility (XV1 of the Tardieu Scale) of the soleus; gastrocnemius; gluteus maximus; and rectus femoris muscles; daily time of self-stretch posture for each of these muscles; maximal ambulation speed.	Guided Self-rehabilitation Contract plus conventional therapy	conventional therapy	IG 52 weeks (1 session per week + 3 sessions per week); CG 52 weeks (3 sessions per week)	Conventional therapy	Pradines M, Ghedira M, Portero R, Masson I, Marciniak C, Hicklin D, et al. Ultrasound Structural Changes in Triceps Surae After a 1-Year Daily Self-stretch Program: A Prospective Randomized Controlled Trial in Chronic Hemiparesis. Neurorehabil Neural Repair 2019;33:245–59. https://doi.org/10.1177/1545968319829455.
2019	Saadatnia	Iran	x			29	28		Barthel Index Score; Modified Rankin Scale; Fugl-Meyer Upper and Lower Extremity Sensorimotor Score;	Home-based rehabilitation exercise program which was recorded on DVD	usual care	12 weeks (not specified)	Usual care	Saadatnia M, Shahnazi H, Khorvash F, Esteki-Ghashghaei F. The Impact of Home-Based Exercise Rehabilitation on Functional Capacity in Patients With Acute Ischemic Stroke: A Randomized Controlled Trial. Home Health Care Manag Pract 2020;32:141–7. https://doi.org/10.1177/1084822319895982.
2019	Sczesny-Kaiser	Germany			x			18 (9/9)	10-meter walking test; timed-up-and-go test; 6-min walking test; functional ambulatory category; Berg-Balance Scale.	hybrid assistive limb-assisted, supervised, body-weight supported treadmill training	Conventional Physiotherapy	6 weeks (5 sessions per week; 30')	Conventional physiotherapy	Sczesny-Kaiser M, Trost R, Aach M, Schildhauer TA, Schwenkreis P, Tegenthoff M. A Randomized and Controlled Crossover Study Investigating the Improvement of Walking and Posture Functions in Chronic Stroke Patients Using HAL Exoskeleton – The HALESTRO Study (HAL-Exoskeleton

														STROke Study). Front Neurosci 2019;13:259. https://doi.org/10.3389/fnins.2019.00259.
2019	Sheng	China		x	x	30	30		10-Meter Walking Test; Timed Up and Go Test; stride length; stance phase; swing phase; foot rotation of the involved side.	routine rehabilitation plus kinesio taping treatment	Routine rehabilitation	Not specified	Routine rehabilitation	Sheng Y, Kan S, Wen Z, Chen W, Qi Q, Qu Q, et al. Effect of Kinesio Taping on the Walking Ability of Patients with Foot Drop after Stroke. Evid Based Complement Alternat Med 2019;2019:1–7. https://doi.org/10.1155/2019/2459852.
2019	Stolz	Australia	x	x		20	20		10-Meter Walking test; gait speed and cadence; 6-Minute Walking Test; Functional Independence Measure; Timed Up&Go Test; EuroQol five-dimension	one-on-one conventional physiotherapy plus gait training using the Cable-Driven Gait Training device	one-on-one conventional physiotherapy	IG (5 days per week; 30' + 30' per session); CG (5 days per week; 60' per session)	Conventional physiotherapy	Stolz R, Nayyar R, Louie J, Bower KJ, Paul SK, Ng L. The effectiveness of a novel cable-driven gait trainer (Robowalk) combined with conventional physiotherapy compared to conventional physiotherapy alone following stroke: a randomised controlled trial. Int J Rehabil Res 2019;42:377–84. https://doi.org/10.1097/MRR.0000000000000375.
2019	Wang	China		x	x	222	222		Modified Ashworth Scale (elbow flexors; wrist flexors; finger flexors; knee extensors; ankle plantar flexors); Fugl-Meyer Assessment; Modified Barthel Index	Tibetan medicated bathing therapy + conventional rehabilitation	Conventional rehabilitation	4 weeks (5 times per week)	Conventional rehabilitation	Wang M, Liu S, Peng Z, Zhu Y, Feng X, Gu Y, et al. Tibetan Medicated Bathing Therapy for Patients With Post-stroke Limb Spasticity: A Randomized Controlled Clinical Trial. J Am Med Dir Assoc 2020;21:374-380.e4. https://doi.org/10.1016/j.jamda.2019.10.018.
2019	Zhang	China	x			48	48		Stroke Neurological Deficit Scores; Fugl Meyer Assessment; Mini-Mental State Examination; Clinical Dementia Rating;	early rehabilitation nursing model	outine precautions, monitoring of vital signs, and rehabilitation exercise guidance	Not Specified	Conventional nursing model	Zhang X, Sheng Y, Gao S. Effects of early rehabilitation nursing on improvement of conditions and quality of life in patients after

									Quality of life evaluation; Nursing satisfaction.					ischemic strokes. Int J Clin Exp Med 2019;12:11412–9.
2020	Ambrosini	Italy	x			34	34		10-meter walking test; 6-meter gait analisys; 6-Minute Wlaking Test; Functional Independence Measure motor subscale; Motricity Index leg subscale; Trunk Control Test; Berg Balance Scale; gait spatio-temporal parameters; Global Perceived Effect	biofeedback training: 15 sessions of cycling with FES, 15 sessions of platform based balance training; plus usual care	Usual care	6 weeks (5 days per week, 90' per session)	Usual care	Ambrosini E, Peri E, Nava C, Longoni L, Monticone M, Pedrocchi A, et al. A multimodal training with visual biofeedback in subacute stroke survivors: a randomized controlled trial. Eur J Phys Rehabil Med 2020;56. https://doi.org/10.23736/S1973-9087.19.05847-7.
2020	Chen	China	x			90	90		Berg Balance Scale; Brunnstrom staging; Fugl-Meyer motor assessment; 10 m walking test; Musculoskeletal ultrasonography (thicknesses of abdominal muscles of the paralytic side)	Core muscle training + routine rehabilitation training	Trunk control training + routine rehabilitation training	8 weeks (6 days per week, once a day for 40'+ 40')	Conventional hemiplegia rehabilitation therapy Routine rehabilitation training	Chen X, Gan Z, Tian W, Lv Y. Effects of rehabilitation training of core muscle stability on stroke patients with hemiplegia. Pak J Med Sci 2020;36. https://doi.org/10.12669/pjms.36.3.1466.
2020	Kumar	India	x			63	70		Short-form 36; MRC of upper and lower limbs; National Institutes of Health Stroke Scale; Mini-Mental State Examination; Modified Ashworth Scale	Robotic tilt-table exercise session plus exercise program for upper extremities	conventional physiotherapy	4 weeks (6 sessions per week, 40' + 15' per session); CG 4 weeks (6 sessions per week, 50'-60' per session)	Conventional physiotherapy	Kumar S, Yadav R, Aafreen. Comparison between Erigo tilt-table exercise and conventional physiotherapy exercises in acute stroke patients: a randomized trial. Arch Physiother 2020;10:3. https://doi.org/10.1186/s40945-020-0075-2.
2020	Lee	Republic of Korea			x	25	24		Posturography (static balance ability); Timed Up and Go test (dynamic balance ability); Functional Reach Test; Berg Balance Scale	Balance training with EMG-triggered FES + general rehabilitation	Balance training without EMG-triggered FES + general rehabilitation	6 weeks(5 days a week; 40' + 60')	General rehabilitation Conventional therapy	Lee K. Balance Training with Electromyogram-Triggered Functional Electrical Stimulation in the Rehabilitation of Stroke Patients. Brain Sci 2020;10:80. https://doi.org/10.3390/brainsci10020080.
2020	Liu	China		x		25	25		Berg Balance Scale; Fugl-Meyer Assessment; Barthel	Sling exercise therapy	Routine physiotherapy	4 weeks (5 sessions per week, 30' per session)	Routine physiotherapy	Liu J, Feng W, Zhou J, Huang F, Long L, Wang Y, et al. Effects of sling

									Index; Short form 36; Shoulder pain VAS					exercise therapy on balance, mobility, activities of daily living, quality of life and shoulder pain in stroke patients: a randomized controlled trial. Eur J Integr Med 2020;35:101077. https://doi.org/10.1016/j.eujim.2020.101077.
2020	Mustafaoglu	Turkey		x	x	IG1: 17; IG2: 17	17		Barthel Index; Stroke Specific Quality of Life Scale; 6-Minute Walk Test distance; Stair Climbing Test; Fugl Meyer Assessment-Lower Extremity; Comfortable 10-m Walk Test; Fast 10-m Walk Test; Borg Rating of Perceived Exertion.	IG 1: robot-assisted gait training plus conventional training; IG 2: robot-assisted gait training.	Conventional Training	6 weeks IG 1 (2 sessions per week; 45' + 5 days per week; 45'); IG 2 (2 sessions per week; 45'); CG (5 days per week; 45').	Conventional Training	Mustafaoglu R, Erhan B, Yeldan I, Gunduz B, Tarakci E. Does robot-assisted gait training improve mobility, activities of daily living and quality of life in stroke? A single-blinded, randomized controlled trial. Acta Neurol Belg 2020;120:335–44. https://doi.org/10.1007/s13760-020-01276-8.
2020	Sharma	India	x	x		47	49		Modified Barthel Index; Fugl-Meyer Assement (UE and LE); Hamilton Depression Rating Scale	Low-frequency repetitive transcranial magnetic stimulation (rTMS) + conventional physical therapy	Sham-rTMS + conventional physical therapy	2 weeks (10 session + 45-50' of conventional physical therapy)	Conventional physical therapy Standard physical therapy Standard care Standard therapy	Sharma H, Vishnu VY, Kumar N, Sreenivas V, Rajeswari MR, Bhatia R, et al. Efficacy of Low-Frequency Repetitive Transcranial Magnetic Stimulation in Ischemic Stroke: A Double-Blind Randomized Controlled Trial. Arch Rehabil Res Clin Transl 2020;2:100039. https://doi.org/10.1016/j.arrct.2020.100039.
2020	Sürücü	Turkey		x		20	20		Modified Ashworth Scale; Range of Motion; Brunnstrom's neurophysiological assessment; Modified Motor Assessment Scale	Electromyography biofeedback therapy + conventional therapy	Conventional physiotherapy	3 weeks (5 days a week, 15 session; 20' + not specifeid amount of time of conventional physiotherapy)	Conventional physiotherapy	Dost Sürücü G, Tezen Ö. The effect of EMG biofeedback on lower extremity functions in hemiplegic patients. Acta Neurol Belg 2020. https://doi.org/10.1007/s13760-019-01261-w.
2020	Swank	USA	N/A	N/A	N/A	37	36		Stroke Rehabilitation Assessment of Movement Measure; Functional	Patient-Directed Activity Program plus usual care	usual care	4 weeks IG (not specified; 180' + 60'); CG (not specified; 180')	Usual care	Swank C, Trammell M, Callender L, Bennett M, Patterson K, Gillespie J, et al. The impact of a

									Independence Measure; Stroke Impact Scale.					patient-directed activity program on functional outcomes and activity participation after stroke during inpatient rehabilitation—a randomized controlled trial. Clin Rehabil 2020;34:504–14. https://doi.org/10.1177/0269215519901153.
2020	Wang	China		x		67	67		Fugl-Meyer Assessment; Barthel Index; 3D Gait Analysis	Acupuncture treatment plus Conventional treatment	Conventional pharmacologic treatment plus physiotherapy	4 weeks (6 days per week; 45' per session)	Conventional treatment	Wang H-Q, Hou M, Li H, Bao C-L, Min L, Dong G-R, et al. Effects of acupuncture treatment on motor function in patients with subacute hemorrhagic stroke: A randomized controlled study. Complement Ther Med 2020;49. https://doi.org/10.1016/j.ctim.2019.102296.
2020	Yadav	India	x			68	65		SF-36; Muscle strength through MRC; National Institutes of Health Stroke Scale; Mini-Mental State Examination; Modified Ashworth Scale.	Erigo tilt-table therapy	conventional physiotherapy	4 weeks (6 sessions per week; 50'/60')	Conventional physiotherapy	Yadav R, Kumar S, Afrin A. EFFICACY OF ERIGO TILT-TABLE AND CONVENTIONAL PHYSIOTHERAPY ON REHABILITATION OUTCOME WITHIN HAEMORRHAGIC AND ISCHEMIC STROKE PATIENTS. J Musculoskelet Res 2019;22:1950003. https://doi.org/10.1142/S0218957719500039.